

Citation: Fulcher v. Conklin, 2013 ONSC 2013

Court File No. 05-CV-293538PD1

Date: 20130408

**ONTARIO
SUPERIOR COURT OF JUSTICE**

B E T W E E N:

EDWARD BRUCE FULCHER, AUSTIN
FULCHER AND NOVITHERM CANADA
INC.

Plaintiffs

- and -

CATHERINE CONKLIN

Defendants

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) *Mr. Alan L. Rachlin* for the Plaintiffs
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) *Mr. David Thompson* for the Defendant
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) **HEARD: Sept. 18-Oct. 11, 2012**

M.A. SANDERSON J.

REASONS FOR DECISION

INTRODUCTION

[1] Mr. Bruce Fulcher ("Fulcher"), born March 24, 1958, now 54 years old, and his son Austin Fulcher ("Austin"), now 21, have brought this action against the Defendant Catherine Conklin (the "Defendant") for damages arising out of a motor vehicle accident on September 20, 2003 (the "accident") when Conklin drove her car into the back of the stopped Fulcher vehicle. Conklin has admitted liability.

[2] While counsel for Conklin has conceded that nine years later, Fulcher continues to suffer from serious back pain and disability, he submitted that they were not caused by the accident. He disputed the extent of Fulcher's continuing disability related to the accident and the quantum of his economic loss.

[3] At the time of the accident, Fulcher owned 35% of the shares of Novitherm Canada Inc. ("Novitherm"), an energy conservation company. Immediately before the accident, Novitherm's revenues had been growing exponentially. Counsel for Fulcher submitted that had the accident not happened, Novitherm would have continued to grow by leaps and bounds and would have yielded significantly higher profits for its two owners, Fulcher and Mr. Frank Snyder ("Snyder") than it has subsequently done. Because the accident compromised Fulcher's unique ability to contribute to Novitherm's growth and revenues, its profits and Fulcher's earnings have been adversely affected.

[4] Counsel for the Defendant Conklin submitted that Fulcher's accident did not adversely affect Novitherm's earnings post-accident to the extent Fulcher and Snyder have claimed. He disputed their evidence and that of the expert called by counsel for the Plaintiff projecting Novitherm profits, Fulcher's share of those profits, and his likely compensation for services to the company had the accident not happened.

[5] I shall later deal specifically with the conflicting evidence pertaining to causation and quantum of damages. Before doing so, I shall outline the background facts.

BACKGROUND FACTS

[6] Fulcher was born in Regina and raised in a small town nearby. He moved to Ontario to attend the University of Waterloo and graduated in 1983 with a Bachelor of Environmental Studies degree.

Pre-Accident Employment

[7] After graduation, between 1983 and 1985, he worked as a commissioned salesman at Save-It, a company selling energy-saving equipment.

[8] He then started his own business, Energy Management Consultants of Canada Limited, advising others on reducing energy costs.

[9] As time passed and Fulcher increasingly worked with two engineers, Messrs. Zelany and Bennett, the three of them formed the firm of Zelany Fulcher and Bennett. However, after the two other principals retired in 1994, Fulcher could not continue in that business as he was not an engineer.

[10] Between 1994 and 1996, among other things, Fulcher did home renovations.

[11] In 1996, he started to work full-time at Novitherm, a company that sells reflectors that reflect heat back into rooms and provide energy savings. At that time, a Mr. Tom Vincent ("Vincent") owned all of the Novitherm shares. Snyder was operating the business on a day-to-day basis.

[12] Fulcher had more technical energy management expertise than Snyder; Snyder had more business and administrative experience.

[13] After Vincent agreed to transfer his shares to Snyder, Snyder promised Fulcher 35% of the Novitherm shares as "sweat equity" if he would work at less than market rates while they were building the business together. They decided to target owners of multi-residential buildings to sell heat reflectors directly to them.

[14] It took time to build up the Novitherm business. While single owners of one or two buildings might buy heat reflectors for installation in those buildings without prior testing, owners of multiple buildings typically required evaluation of the performance of the heat reflectors in one building over a complete heating season before they would agree to install

them in multiple buildings. There would be a time lapse of up to two years from initial contact through testing to installation of reflectors.

[15] Between 1996 and the accident, Fulcher performed multiple roles within Novitherm. He made sales. He prepared case studies and proposals to demonstrate that their heat reflectors could and would produce energy savings. He and Snyder attended industry conferences. He applied his technical expertise, inspecting and adjusting furnaces, boilers, heat conductors [on rooftops, in boiler rooms and elsewhere.] Inspections and taking measurements often necessitated moving furniture and feeling around with his hands: he was required to kneel, bend, twist and climb. He inspected baseboard convectors and other equipment on his knees looking up with his head twisted to the side.

[16] Novitherm had/has no direct competitors in Ontario, Quebec or the Northeastern United States.

[17] Before the accident, in addition to servicing a developing Novitherm business in the GTA area, Snyder and Fulcher had decided that Snyder would open up the Quebec market, Fulcher the Ottawa market.

[18] At the time of trial in 2012, in its primary market in the GTA, Novitherm had penetrated only about 25% of its market potential; Ottawa had not been developed at all.

[19] Fulcher and Snyder gave evidence that in the period immediately before the accident, they were having considerable success. Novitherm's revenues had jumped from \$47,649 in 2000, to \$147,025 in 2001, \$457,460 in 2002, and \$990,683 in 2003 [the last full year before the accident.] They both expressed the view that but for the accident, this positive trajectory would have continued.

[20] As its revenues and income increased, Snyder and Fulcher expected to receive compensation and return commensurate with their ownership interest.

Pre-Accident Activities and Interests

[21] Before November of 2002, Fulcher was an active man.

[22] He loved to ride motorcycles. Between 1992 and 2000, he had been a motorcycle instructor at Humber College.

[23] He was an enthusiastic mineral collector. As a member of the Walker Mineral Club [associated with the University of Toronto and Ontario Museum], he participated in 10-12 field trips per year to collect natural crystals [including quartz, fluorite, pyrite, pink granite and feldspar crystals.] Along with other club members he travelled to abandoned mine sites, road cuts, etc., to extract the minerals, wielding sledgehammers, saws, and other tools and equipment.

[24] As detailed later, he was training in the martial arts. He regularly engaged in other forms of exercise and sports. He was a fisherman.

[25] Fulcher had been divorced some years prior. At the time of the accident, he was involved romantically with Ms. Pauline Lee ("Pauline"), a registered nurse at the Hospital for Sick Children.

[26] In the spring of 2002, Pauline and Fulcher had taken a physically strenuous trip to Vietnam. While she had assisted in surgeries in a Vietnamese hospital, Fulcher had helped keep waiting families organized, etc. Before returning to Canada, they had travelled and hiked.

[27] Fulcher's son Austin, 12 at the time of the accident, lived with him about half of the time. Now a fourth year environmental engineering student at the University of Guelph, he gave evidence that prior to November 2002, he and his father engaged in a wide array of activities together, including soccer, baseball, fishing, mineral collecting, motorcycle riding and jiu-jitsu, a martial art. Every summer they took fishing trips to a family-owned fishing camp 8 hours north of Regina. Every September they went salmon fishing in Ontario in the Pine River.

Health Prior to Date of Martial Arts Injury – November 2002

[28] Fulcher gave evidence that apart from a problem with his parotid gland that required periodic surgery; a childhood high-jumping accident; some temporary sequelae from over-exposure to carbon monoxide in 1999; and some back stiffness, his health was unremarkable prior to November 2002.

[29] Dr. Green, Fulcher's family doctor since 1998, gave evidence. In a note dated December 12, 1998, Dr. Green noted "chronic neck and back pain x years." However, his office notes before November 2002 contain little mention of back pain.

The Martial Arts Injury – November 2002

[30] Fulcher's jiu-jitsu injury in November 2002 is central to the dispute about causation in this case.

[31] Fulcher gave evidence that before the jiu-jitsu incident ("the incident"), he was spending 5-6 hours per week at a local dojo.

[32] He described the incident and its immediate sequelae as follows:

I had a guy in the corner. He was a lot bigger than me. So I was happy to have him cornered and I went to give him a roundhouse kick ... to the right side of the head with my left leg and I got it up to above his shoulder and something went 'click' in the back. I ended up kicking him in the shoulder instead. ... I just got sore for a couple of weeks ... After a couple of weeks I started getting shooting pains down my legs and that is what I imagined being stunned by a taser would be like ...

[33] Pauline gave evidence that the night of Fulcher's martial arts injury, he was favouring his back. The next morning he was uncomfortable. To avoid aggravating his back, he reduced his physical activities and took medications as prescribed. By Christmas of 2002, he was improving.

[34] Dr. Green's office note dated April 22, 2003 contains the following: "Back pain 1+ tender low pain..."

[35] Dr. Green's May 12, 2003 office note includes the following: "Still back pain and iliac crest. Wakes up with pain. Lasting 4-5 months. Started with kick-boxing injury. Diagnosis ? disc."

[36] In April or May of 2003, Dr. Green ordered a CT scan of Fulcher's back and referred him to a neurologist, Dr. Shiffer.

[37] The report on the CT scan dated May 30, 2003 mentions a 6mm herniated disc at L4-5 in the left anterior aspect of the spinal canal, indenting the adjacent dural sac and nerve roots.

[38] Dr. Shiffer did not see Fulcher until July 2003.

[39] Fulcher gave evidence that by the time he first saw Dr. Shiffer, his back was improving. The intensity and frequency of the sciatica were reducing. The only medication he was taking was over-the-counter Tylenol.

[40] Dr. Shiffer gave evidence that when she saw Fulcher in July 2003, he was complaining of back pain. On examination, his bulk muscles were normal in tone. He had decreased knee-jerk on the left that indicated to her that his fourth lumbar nerve root might not be functioning normally. She noted some positive straight-leg raising.

[41] Dr. Shiffer's evidence contains the following:

Q. And Doctor, in the spectrum of the patients you see with these sorts of problems, how serious were the problems that you found Bruce to have when you saw him in July of 2003?

A. Not terribly serious in the sense that while he was symptomatic and had pain, he did not – and he had maybe a little bit of sensory difficulty, he wasn't having florid weakness. He was still ambulatory. He was able to walk. He wasn't on heavy duty pain medication. He did not have any bowel or bladder disturbance. So, you know, he was uncomfortable in a fashion but is very common with a common condition which is a disc disease.
[Emphasis added.]

[42] Dr. Shiffer felt his back problems could improve and resolve with time, and recommended a course of conservative therapy, including stretching of the spine. She did not refer him to a surgeon.

[43] Fulcher gave evidence that between the time he first saw Dr. Shiffer in July and the accident, his condition continued to improve. Although he was not completely pain-free, he could engage in many of his normal activities. His back problems from the jiu-jitsu incident were not interfering with his ability to do laundry, change the bed linens, mop wood floors, vacuum the carpet, clean the kitchen and bathrooms, mow the lawn or plant and tend the garden. He was teaching Austin to cook.

[44] Pauline gave evidence that by the summer of 2003, Fulcher was gardening. He was working on a daily basis. They were taking day trips together, often involving long periods of sitting in the car. They enjoyed cooking together. His sleeping patterns were normal.

[45] In early September 2003, he was typically working about 60 hours a week. When Austin wasn't living with him, he worked longer hours, including weekends, unless he was

mineral collecting. In addition to selling, inspecting equipment and preparing proposals in the GTA, in the few months preceding the accident, he was frequently travelling to Ottawa on Novitherm business.

[46] Austin said Fulcher was getting better before the motor vehicle accident in September 2003. He and his father had started kicking a ball around together. In August of 2003, his father was feeling well enough to drive on gravel roads to the fishing camp 8 hours north of Regina. He was doing their laundry, mopping, vacuuming, caring for the lawn, cleaning the bathroom, and shopping for groceries.

The Accident: September 20, 2003

[47] On September 20, 2003, while Fulcher was stopped at a red light in the northbound lane of Kipling Avenue near the Queen Elizabeth Way and Austin was a passenger in the front seat, he heard screeching from behind and turned his body to the right towards Austin. His foot was on the brake pedal. A green Volvo driven by the Defendant Conklin smashed into the rear end of their vehicle. He felt "quite the jolt." They were pushed forward approximately 30-40' into the intersection.

[48] In her Collision Reporting Form, Conklin reported that she had failed to notice a red light and her car had struck the Fulcher vehicle at 35-40 kph. Her cost of repairing the damage to her vehicle was \$13,663.66.

[49] Fulcher said after he checked to make sure Austin was all right, he decided to check on Conklin. When he stood to get out of his car, his lower back was much sorer than it had been for a long time.

[50] Pauline said on the night of the accident, Fulcher was complaining of severe pain in his back, going down his legs. She had to help him roll over to get out of bed. His movement was greatly diminished. He had difficulty climbing and descending stairs.

[51] Fulcher's evidence includes the following:

Q. And how did your health change over the weeks following the car accident?

A. It was a huge setback. I had more and more aggravated back pain in the weeks following, which then again turned into sciatica, i.e., shooting pains down the legs.

Appointment with Dr. Shiffer on October 29, 2003

[52] After the accident, Fulcher kept the appointment for October 29, 2003 he had earlier scheduled with Dr. Shiffer. He said pre-accident he had been hoping she would give him a clean bill of health so he could resume martial arts and motorcycle riding.

[53] Dr. Shiffer's evidence about her October 29 examination of Fulcher [about five weeks after the motor vehicle accident] included the following:

Q. So what were your findings on examination of Bruce that day?

A. ... his neurological examination was essentially unchanged from previous. He was still normal in his strength. The only difference, however, is that ... he was now extremely uncomfortable moving, so getting on and off the examination table and he was now walking with what we call an antalgic gait, meaning limping, because you're uncomfortable. And straight-leg raising I now describe as

extremely positive. So when I say that, it usually means that if I try and lift someone's leg on the examining table, they're kind of on the ceiling with discomfort.

Q. ...What in your mind were the differences in your examination of Bruce between the two visits?

A. The main difference really is that he was manifesting the behaviour that indicated he was markedly in pain and that was now affecting his [ability] to walk and move comfortably.

Q. And what did you do or what did you conclude as to what was going on with Bruce's back as a result of the history you took and the examination you did on October 29, 2003?

A. Well, my conclusion was that he likely was again having symptoms from compression of either L4 or L5 nerve root because of a disc at the L4 or 5 interspace, and that he had now failed conservative therapy. It had been some time since I had seen him previous and instead of getting better, he was now worse and he was sufficiently uncomfortable and that discomfort was not being managed by pain medication. So I felt we needed to get more detailed imaging to look and see whether anything had changed anatomically speaking at that level in his back. And I also did think, given that he was now extremely uncomfortable, despite physio, that he might need back surgery and I wanted the opinion of a neurosurgeon.

[54] Dr. Shiffer gave evidence that she concluded his neurological condition on October 29 was worse than it had been in July:

Q. And the neurological signs, to the extent that he had neurological signs, were worse when you saw him in October than they had been or as compared to July 2003?

A. I would say yes.

Q. What if anything does that say to you as a neurologist about the extent to which the disc might have been irritating or compressing a nerve root between the two?

A. I would assume from that that his nerve is now more compressed and more irritated.

[Emphasis added.]

[55] In addition to arranging an MRI of his back, Dr. Shiffer referred Fulcher to a surgeon, Dr. Schutz.

[56] Dr. Green said that when he examined Fulcher on November 11, 2003, his level of pain was "much worse" than it had been before the accident. He prescribed Percocet, a very strong pain medication.

[57] Austin gave evidence that by a few weeks after the accident, his father's ability to do most things had severely declined. His concentration had deteriorated. He moved more slowly. He winced when standing. He had difficulty getting out of the car. Because he couldn't concentrate, he didn't even participate in non-physical activities such as playing Scrabble.

[58] Fulcher said as time passed, it became more and more difficult for him to function. He had pain sitting and standing. He was "rigid as a pipe." He couldn't bend over or move laterally. He was able to do "less and less" at Novitherm. His physical problems, primarily his back problems, compromised his ability to examine, measure and adjust heating units. He had difficulty bending, kneeling, twisting and climbing. Although he tried to work as before, his concentration, physical capability and stamina were seriously compromised. His cognitive problems affected his ability to prepare case studies [to demonstrate energy savings from the use of Novitherm's heat reflectors.] His cognitive and physical deficits combined to render him less able to develop the GTA market as before and unable to develop the Ottawa market at all.

[59] Because of Fulcher's injuries, Snyder had to do much of what Fulcher had previously done. Snyder had no time to develop the Montreal market.

[60] Both Snyder and Fulcher gave evidence that the overall result of the accident to Novitherm was a substantial decrease in its revenues [compared to what they would have been had Fulcher not been injured.] Momentum, once lost, was difficult to restore.

[61] Dr. Green noted on January 19, 2004 [i.e., even before Fulcher saw Dr Schutz] that "his work down 50%" since accident.

[62] As noted earlier, there was a three month time lag between Dr. Shiffer's October referral to Dr. Schutz and Fulcher's first appointment with him on January 22, 2004. Dr. Schutz concluded that Fulcher urgently needed decompression surgery.

[63] Dr. Schutz's consultation note dated January 22, 2004 contains the following:

Now his back pain is exceptionally severe and even since December, things have gotten out of hand. He cannot move very well without pain radiating into his legs. The left side is now worse than the right, but before it was bilateral. Coughing and sneezing sends him into orbit. He started the usual treatment all to no avail. A recent MRI scan shows a very large disc herniation at L4-5 to the left.

[64] Five days later on January 27, 2004, Fulcher had decompression surgery.

Post-Surgical Developments

[65] Fulcher gave evidence that "even before the stitches were out, back spasms started." He said, "It's pain like you never felt before... the jiu-jitsu incident was absolutely no comparison whatsoever." The back pain was "intense. 24 hours a day."

[66] From the time of his back surgery in early 2004 until the beginning of 2005, he was working as much as he could, but "just a fraction of what I did before."

[67] On February 27, 2004, four weeks post-surgery, Dr. Green noted that he had physiologic scoliosis with spasm.

[68] Fulcher's evidence includes the following:

A. It was a really rough time. The pain was overwhelming and it really wears on you. It prevented me from getting much sleep at all. I would venture to guess I didn't get more than 2 hours of sleep straight in a night for all those years. Probably four hours cumulatively. So that wore me down a lot. The pain was so bad that if it wasn't for my son, I probably would have just killed myself.

[69] He said he was stiff. He couldn't bend or move his neck side to side, move his ear to his shoulder, twist or turn. Over time, he developed bloody nightmares two, three, four, five times per night. The dreams, with a theme of helplessness, started nicely but ended horrifically.

[70] Austin said when he would get up in the middle of the night, his father would be up.

[71] Pauline said Fulcher could not sleep at night. He was sleep-deprived on a daily basis. He tried to do some things like cooking, grocery shopping and laundry. He couldn't carry anything heavy.

[72] In April of 2004, Snyder wrote a letter to Fulcher containing the following: "At best you are performing at 25% of your abilities that you were able to provide prior to your accident ..."

[73] On May 5, 2004, Dr. Clements, an orthopaedic surgeon, examined and interviewed Fulcher in an independent orthopaedic examination at the request of the accident benefits insurer. He prepared a report including the following at p. 4: "...His body movements were guarded and his spinal rhythm altered. Left and right lateral flexion was approximately 1/3 the expected norm to the left and to the right." At page 9: "Mr. Fulcher has demonstrated a reduced active range of movement in his lumbar spine and would have reduced capacity for lifting, bending, stooping, prolonged sitting and standing or carrying heavy objects ..."

[74] Dr. Clements noted that Fulcher was working 6-8 hours per week at his occupation.

[75] By June 2004, Dr. Green noted that Fulcher was seeing a psychiatrist for depression. [Between July 2004 and May 2005, Fulcher received counselling from Dr. Bernstein.]

[76] On October 10, 2004, Dr. Green noted Fulcher was taking physiotherapy three times per week and only working part-time.

[77] Dr. Angela Mailis-Gagnon, a physical medicine and rehabilitation specialist, examined Fulcher on March 18, 2005. She noted he was suffering from moderate to severe pain and having bizarre episodes of very severe pain. On clinical examination, he was suffering from back pain and limping, stiff and flushed (an indication of pain). He had reduced range of motion in the neck. Most of her findings concerned his back. She said she felt spasm [an objective sign.] Fulcher struck her as "stoic" and "genuine." She prescribed heavy duty painkillers, sleeping pills and a topical ointment to reduce the pain.

[78] Dr. Mailis ordered another M.R.I. on June 10, 2005.

[79] On June 20, 2005, Dr. Clements saw Fulcher again. Fulcher was describing ongoing back pain radiating across the lower back diffusely from the mid-thoracic region to the lower back. Left and right lateral flexion of the lumbar spine were estimated at "half the expected normal."

[80] Dr. Clements' June 2005 report includes the following:

At page 7:

The surgical procedure indicates that a facet joint was taken down at L4-5 to gain access to the disc ... This would also contribute to ongoing symptoms of mechanical instability in the lumbar spine ... It is my estimation that he in essence has achieved a maximum level of medical recovery. I anticipate that Mr. Fulcher will have residual limitations with regard to his lumbar spine. ... With regard to standing, sitting, bending, lifting, pushing and carrying ... it is conceivable that Mr. Fulcher's mechanical symptomology may increase as he ages and may require further investigation and potentially surgical treatment in the future.

[81] On July 26, 2005, Dr. Green noted "low back pain, physiotherapy three times a week and laser." He noted that Fulcher was working 50% of the time.

[82] When Austin started high school in September 2005, he wanted to attend Humberside Collegiate. During the summer of 2005, in order that Austin would qualify, Fulcher purchased a former rooming house at 204 St. John's Road.

[83] Fulcher gave evidence that although he had previously done renovations to his own homes and as a means of earning a living, his back injuries limited his capacity to renovate that house. He was able to do some light work, including installing some 2x4s, some 38" high wainscoting, some baseboards and some tile, and removing some cupboards. With Austin's help, he was able to paint the living room, den, spare bedroom, and second floor bedroom. However, he could not install new cupboards, drywall the ceiling, or plaster or paint the ceilings. He hired an electrician and a plumber to do work he would have done himself before the accident.

[84] On November 11, 2005, Dr. John Newell, orthopaedic surgeon, interviewed and examined Fulcher. He gave evidence that his main physical problems at that time were muscle spasms and severe constant low back pain, aggravated by stooping, sitting for extended periods and forward flexion. On examination Dr. Newell noted a flattened back, which he said was indicative of spasm in the paravertebral muscles of his lumbar spine [an objective sign.] He diagnosed myofascial injury of Fulcher's cervical spine, aggravation of pre-existing degenerative disc disease in his lumbar spine and probable aggravation of his L4-5 disc herniation.

[85] Dr. Newell noted Fulcher was having difficulty sleeping because of back pain, nightmares and feelings of helplessness. He felt Fulcher was genuine.

[86] On December 6, 2005 in his office notes, Dr. Green noted continuing back pain.

[87] On December 19, 2005, Dr. Green noted lots of spasm in the back.

[88] On December 25, 2005 Dr. Green noted low back pain, spasms, stiffness in back and neck.

[89] On January 26, 2007, Dr. Green noted Fulcher was sleeping poorly.

[90] On April 11, 2007, Dr. Green noted "Spasm – low back."

[91] Fulcher said over time, his back pain worsened.

[92] Pauline gave evidence that her relationship with Fulcher ended in 2007 because of his irritability. He never regained the ability to sleep through the night. He never accompanied her on another medical mission. He couldn't have managed a long plane trip.

[93] On June 13, 2011, Dr. Newell saw Fulcher again. On examination, he again found back spasm. He said because it had been necessary for Dr. Schutz to remove part of the facet joint [an integral part of the vertebrae in the motion segment, like part of the knee joint] as well as to remove ligament and bone and part of the disc, the surgery had caused segmental instability, a condition associated with pain with activity and chronic back pain that never resolves.

Fulcher's Condition at the Time of Trial

[94] Fulcher estimated that at the time of trial, he was 20-25% as productive at Novitherm as he had been pre-accident.

[95] At the time of trial, he said he "always" had back pain. However, its severity had diminished, was "a little less" than earlier. He attributed that diminishment to the fact that at least two times per week during the two years preceding the trial he had been working with personal trainers to strengthen his upper body and legs and had done floor, bench and free weights work to strengthen his core/support his back muscles. Three times a week since May of 2011, he had also been working with a yoga instructor. These activities have made him feel stronger and have reduced the level of his back pain.

[96] Fulcher gave evidence that his continuing disabilities stem from a combination of physical and cognitive deficits. His powers of concentration have been compromised since the accident. His ability to prepare proposals for potential customers has been diminished. He makes mistakes he did not make before. "My mind is like thinking through mud. I attribute it to constantly being in pain. It's hard to focus on things and hardly ever getting any sleep is probably the real dominant reason why. ... It affects my ability to think quickly on my feet, to prepare sales presentations and analyses, paybacks, proposals with respect to multi-residential buildings."

[97] He said on a good day he can function reasonably well, "the mud isn't as thick." He has 1 or 2 "good days" per week and 1 or 2 "bad days" per week. The rest are "somewhere in between."

[98] Austin gave evidence that although his father's emotional state deteriorated after the accident, causing him to be short of patience "because of the pain," in the last couple of years it has improved. However, he continues to be frustrated about his cognitive difficulties, including his inability to remember names. He is good at putting on a brave face. However, he has good, bad and average days. He still has difficulty moving, lifting, bending and stretching. He has not resumed martial arts. They have not fished together since the accident. [Fulcher's back cannot take jarring in a truck all the way up to the cabin 8 hours north of Regina.] They have not been motorcycling. They cannot play sports together.

[99] As Field Coordinator for the Walker Mineral Club, Fulcher has been able to return to some mineral collecting activities. He hasn't been able to use a sledgehammer or gas powered stone saw since the accident. He cannot stand for lengthy periods. He must choose locations where he can sit and scrape for minerals. He said as a result, he can't find specimens that are as good as before.

[100] Mr. Steede gave evidence that he has known Fulcher for about 25 years through the Walker Mineral Club. Prior to 2002 he was not aware Fulcher had any problems with his back. He was aware of the martial arts injury and had been under the impression that that had been healing before the accident. He said in 2009, after Fulcher resumed some of his mineral collecting activities, he went with Steede and other Club members on a four-day collecting trip to Gatineaus. During the first three days of mineral collecting, Fulcher didn't use a sledgehammer. The primary activity, a "last in a lifetime opportunity," at a working mine,

was scheduled for the fourth day. Fulcher could not participate at all because his back was too sore.

EVIDENCE SPECIFICALLY RELEVANT TO DISPUTED FACTS

Disputed Facts re Causation

(a) Did This Rear-End Collision Cause or Aggravate Fulcher's Pre-Existing Disc Herniation?

[101] Counsel for the Defendant relied on the evidence of Dr. Paitich [a specialist in orthopaedic surgery with a sub-speciality in the spine, who saw Fulcher on December 16, 2010 at the request of counsel for the defence] in chief that the accident did not cause Fulcher's disc herniation.] He opined that rear-end collisions cause extension, not flexion, injuries. Flexion injuries, not extension injuries, cause disc herniations. Fulcher did not have a flexion injury here because his car was hit from behind.

[102] Counsel for the Plaintiff relied on the evidence of Dr. Newell that motion studies have demonstrated that in rear-end collisions, bodies are thrown forward and backward, not just once but many times. His clinical experience over the years has been that patients have sustained lumbar disc injuries in rear-end collisions.

[103] I note that Dr. Paitich conceded in cross-examination that whiplash injuries are generally characterized as flexion/extension injuries. In a rear-end collision, a body is pushed backward causing extension [i.e., the neck and back move back, then the head goes forward causing flexion.] Dr. Paitich agreed that in the accident, Fulcher's lumbar spine likely extended, then flexed.

[104] Counsel for the Plaintiff [relying on the evidence of Drs. Shiffer, Newell, Mailis and Clements] submitted more generally that the accident worsened Fulcher's pre-existing disc herniation and necessitated the January 2004 surgery.

[105] In examination-in-chief, Dr Newell's evidence contains the following:

Q. Is a person with a previously herniated lumbar disc at any different risk of further injury in a rear end collision as compared to a healthy member in the population?

A. Yes, in my opinion.

Q. Why is that?

A. He already has a disc herniation. He's subjected to forces on his lumbar spine, which caused probably an increase in the disc herniation itself but I think we have to concentrate on the facts. The fact he was functioning normally up until the time of the accident. After the accident he developed leg pain. ... surely Dr. Schutz would not have operated unnecessarily unless he thought it was necessary.

[106] In her report dated May 2005, Dr. Angela Mailis Gagnon opined as follows:

From a predisposing point of view, Mr. Fulcher had long-standing degenerative disc disease and a moderate disc herniation at L4-5... However, while he had reduced his leisure personal fitness activities he was able to perform his occupation ... From the precipitating point of view, I have enough documentation to suggest that Mr. Fulcher became much worse ... Dr. Shiffer's note in October 2003 compared to her July 2003 note and also that this gentleman has been genuine and forthcoming. Therefore I feel the accident materially contributed to the worsening of his symptoms. From the perpetuating point of view, Mr. Fulcher continues to suffer from fluctuating and substantial

back pain (? Instability) While his sciatica is much improved after the surgery, his adjustment disorder is also another factor, secondary to unrelieved pain.

[107] Dr. Newell agreed with Dr. Clements' assessment that the accident had provided a quantum of energy to his lumbar spine sufficient to increase his symptoms and perhaps worsen his herniation. He said:

It was my opinion at the time he probably sustained a myofascial injury of his neck, myofascial injury of his lumbar spine and a recurrent aggravation of disc herniation.

...He had an obvious injury to his back at that time [November 2002] with a probable disc herniation recurring at that time. However, his symptoms seemed to resolve and he was able to do all his usual day-to-day activities until the car accident. After the car accident, he had increasing back pain, especially he had leg pain. He eventually saw Dr. Schutz and Dr. Schutz felt that his pain was sufficient to warrant an operation ... he was functioning quite well until his accident of 2003. He's doing all his usual day to day activities. He was working 60-70 hours a week and after the car accident he had acute back pain and particularly leg pain sufficient to warrant operation. In my opinion if it had not been for the accident, it is unlikely he would have needed an operation, at least at that time.

[108] Dr. Shiffer said a person with a bulging or herniated disc is more vulnerable:

Q. In your experience, what kinds of things cause a worsening of discs that are bulging or herniated?
A. Many things can worsen disc pain. Really, any kind of strain, any kind of injury in my experience can sometimes worsen people's pain. Sometimes it's as trivial as someone coughing, bending the wrong way, sneezing. Sometimes it's impact... a trip and fall, lifting something too heavy, a traumatic injury.

Q. What do you mean by a traumatic injury?

A. ...Any kind of situation where a body may be thrown, jarred, subject to a sudden force or impact.

Q. And have you experienced patients who sustained these sort of injuries after car accidents?

A. Yes.

Q. And is a person with a bulging or herniated disc any more vulnerable to further bulging or herniation than would be someone who didn't have that problem?

A. Yes. I would say they probably are. You know, you can imagine you already have a little bit of tissue, a little bit of disc material right up against a nerve fiber. We're talking millimetres of movement that are required. Improvement in pain can mean that that disc fragment backing off a nerve literally a couple of millimetres. And worsening of pain can mean movement of that disc material a couple of millimetres. That may be all it takes to have intractable [unmanageable] discomfort.

(b) Would Fulcher Have Needed Surgery in January 2004 In Any Event? If not in January 2004, When?

[109] Dr. Paitich agreed with the other experts that the surgery in January 2004 was the cause of Fulcher's ongoing pains. Therefore, his need for the surgery at that time and its likely timing but for the accident is relevant.

[110] There was conflicting evidence as to whether Fulcher would have needed the surgery he had in January 2004 but for the accident and if so, when.

[111] Clearly Fulcher had a vulnerable back even before the accident. The experts agreed that even if the accident had not happened, at some point in the future Fulcher would likely have needed the surgery he had.

[112] Dr. Paitich gave evidence that Fulcher would have needed the surgery in January 2004 regardless of the September 2003 accident. He said in his experience, if Fulcher's disc herniation had been going to resolve without surgery, it would have done so within six months [i.e., before July 2003]. At the time of Dr. Shiffer's neurological consultation in July, Fulcher was already a candidate for disctomy. Neurologists typically refer patients such as Fulcher to a surgeon at or after six months. Dr. Paitich based his opinion on the following. Three months before the accident, a CT scan showed a disc herniation. The M.R.I. taken in December of 2003 confirmed the pre-existing herniation. He assumed there was no change in Fulcher's neurological condition between July 2003 when Dr. Shiffer saw Fulcher before the accident, October 29 when she saw him after the accident.

[113] Dr. Newell gave evidence that given his active lifestyle, even if the accident had not happened, Fulcher would probably eventually have needed surgery in the fullness of time. However, "he would not have required surgery soon after were it not for the accident itself." He first said five, ten or fifteen years after the accident, but then estimated that Fulcher would not have needed that surgery until 20 years from the time of his martial arts injury in November 2002 (i.e., in November 2022 [when Fulcher would be 64.5 years of age.]

(c) Did the Accident Cause Fulcher's Ongoing Physical Impairments? If so, What Physical Impairments?

[114] On May 5, 2004 and June 20, 2005, Dr. Clements noted Fulcher's reduced range of movement and concluded he would have residual limitations with regard to his lumbar spine including limitations standing, sitting, bending, lifting, pushing and carrying.

[115] On June 13, 2011 and November 11, 2005, Dr. John Newell, orthopaedic surgeon, on examination of Fulcher's back, found spasms. The surgery had caused segmental instability, a condition associated with pain with activity and chronic back pain that never resolves.

[116] I have already quoted Dr. Shiffer's evidence that in her opinion, his condition/ neurological signs were worse when she saw him in October than they had been in July 2003. She concluded his nerve was more compressed and more irritated after the accident.

[117] I have already set out the evidence of Dr. Green, the family doctor and of Fulcher, Snyder, Austin, Pauline and the other lay witnesses that his problems with pain radiating into his legs after the martial arts injury had largely resolved prior to the accident, but that he had sciatica within days after the accident that was much worse than he ever had after the martial arts injury.

[118] I have noted his evidence that at the time of trial, he had to pick "a good day" to get out to measure apartment buildings and prepare proposals. On "a bad day," he is not able to work. He can't move his head to check his mirrors. He wants to have a hot bath and crawl into bed with a handful of Tylenol.

[119] Dr. Paitich said he found no evidence of spasm on examination. He opined that Fulcher was "faking"/exaggerating his physical symptoms. He based his opinion in part on Fulcher's report of discomfort on simulated rotation. He said that patients who can raise their legs 90° when seated should be able to raise them 80-90° when supine. When seated, Fulcher

could straight-leg raise 90°, but when lying down claimed to be able to raise them only 45°. This was an "inconsistency." When Fulcher was sitting up, he could rotate his head to the left and right 43°. However, when he was lying down, he could rotate his head 60° to the right. Dr. Paitich said this was another "inconsistency."

(d) Did the Accident Cause Ongoing Cognitive Impairments and Disabilities? If so, What Cognitive Disabilities?

[120] The evidence was conflicting as to the effect of the accident on Fulcher's present and future cognitive functioning. Counsel for the Plaintiffs relied on the evidence of Drs. Levitt and Kaplan to the effect that Fulcher suffers from genuine cognitive deficits resulting from the accident.

[121] In addition to calling Dr. Levitt to give evidence, counsel for the Plaintiff filed the joint report of Drs. Levitt and Kaplan, dated July 10, 2012, dealing with their testing to determine the extent of Fulcher's post-accident cognitive deficits.

[122] Their report contains the following at page 27:

Mr. Fulcher reports chronic and intense back pain that limits him in all areas of his life. Emotional factors play a significant role in exacerbating his pain and as such, we diagnose a pain disorder associated with both psychological factors and a general medical condition. Mr. Fulcher's chronic back pain leading to losses in functioning and in his roles ... major depression which appears to be in partial remission ... in combination with depressive symptoms and chronic pain, there has been a significant impact on reducing this man's cognitive efficiency. We do not diagnose a cognitive disorder as his cognitive impairments do not appear to be related to any injury or disease but rather to the impact of emotional and pain factors. (at p. 33) Cognitive impairments appear to be the result of depression, traumatic distress and pain. We are of the opinion that the September 20, 2003 MVA has caused the psychological disorders he presents with currently. (at p. 36) Cognitive impairments appear to be the result of the interaction of chronic pain and emotional factors. He has attempted to get the appropriate psychological treatment but has been frustrated in his efforts to obtain it. His psychological condition has somewhat improved but remains significant and entrenched. Chronic pain will continue to serve as a factor that makes his psychological state vulnerable. His prognosis for a complete recovery from a psychological perspective is guarded. We are of the opinion that he has sustained a serious and permanent impairment with respect to psychological functioning. [Emphasis added.]

[123] Dr. Levitt concluded Fulcher was genuine and was not exaggerating. His evidence at trial included the following:

Q. All right. So what, if anything, do you take from the combined results of the four subtest we've talked about, the ICN, the INF, the NIM and the PIM for Bruce.

A. On first blush looking at those scales before looking at the other validity scales what I take it this is a pretty clean profile is someone where they should be in terms of impression management if they have psychological issues. So I take from it that he is not exaggerating and trying to get away with something.

...

Q. ... So what does the malingering index value of one tells you?

A. First of all it, tells me that Mr. Fulcher is not likely to be malingering. ...

Q. What does a score of one on the malingering index tell you about Bruce?

A. Tells me that, essentially, he's not likely to be -- to be faking.

...

A: ... on those F scales those need to be elevated up around 90 for you to be concerned about them. If it's not a forensic setting, they could be a little lower, but this was specifically generated with the population in mind of people who are in a legal action.

Q. Say over 90 would be?

A. Over 90 you'd be thinking this person is exaggerating something. You may not know why, but you'd be thinking they're exaggerating something.

Q. And Bruce's scores were 61, 59 and 48?

A. Yeah, those are clean.

[Emphasis added.]

[124] Dr. Levitt's evidence at trial includes the following:

Re Diagnosis

Q. ... Axis 1 ... all of the mental and behavioural diagnoses other than personality disorders and what used to be referred to as mental retardation ... Did you come to any diagnoses for Bruce with respect to Axis 1?

A. I did. ... Those diagnoses were pain disorder associated with a general medical condition and psychological factors, a major depression, ... mild in partial remission and also a complex traumatic stress disorder.

Q. Now those are three different diagnoses?

A. Yes, they are.

Q. Where do those diagnoses come from where ...?

A. The first two come out of the DSM, which is the Diagnostic and Statistical Manual of Mental Behavioural Disorders.

...

A. The third category is kind of a hybrid. Someone has a medical condition or disorder. In this case, with Mr. Fulcher, we know that he had a back injury and he had -- he had problems following surgery. And those are the areas that he complains of still having pain in. So it's not some place else. And psychological factors play a significant role. The way the diagnosis works, they have to play a significant role either triggering the pain, keeping the pain in place or making it worse. And what Mr. Fulcher tells me is that ...when he is under stress, it's like fuel to -- to the fire. ... He also tells me that if he's in a noisy environment and he's having trouble coping with that the pain ramps up higher. So again significant psychological factors that end up worsening his pain. Real pain. This diagnosis doesn't mean he's making it up, but the psychological factors actually make his pain experience in my experience as a rehab psychologist much harder to treat.

Q. Do you have an opinion whether the September 20th, 2003 motor vehicle accident caused or contributed to Bruce's pain disorder associated with the general medical condition and psychological factors?

A. I believe that it did contribute to it, yes.

Q. And then you talked about major depression mild in partial remission. What's -- tell us about that diagnosis, doctor?

A. ...I don't believe that he's got a full blown depression at this point. I think a lot of the depression -- depressive symptoms have come down compared to what they had been. But the depression was there in a full diagnosable state at one point. There's still significant depressive symptoms. He still has suicidal thoughts. Still has struggles with self-esteem. Still has the struggles with motivation. Basically, not enough to qualify anymore as a full disorder, so I classified it as in partial remission, meaning ... it's partly gone. And then mild, meaning that these are on the lower end of -- the number and severity of symptoms that one might see in a major depression.

Re Prognosis

Q. ... One at a time, dealing first with the pain disorder, do you have an opinion as to the likely prognosis for Bruce with respect to that diagnosis?

A. The prognosis would be at the best guarded. Pain disorders are notoriously hard to treat when -- especially when there are psychological factors that are -- that are involved, so I would expect a difficult -- difficult road with that.

Q. So when you say again guarded just, so that the jury understands what you mean by that, can you help us what is your opinion with respect to whether Bruce's likely to get appreciably better in the future insofar as the pain disorder is concerned?

A. I don't see from a psychological perspective the likelihood of significant improvement with respect to the pain disorder that includes psychological factors.

...

Q. Did you find any evidence in the records that you reviewed that Bruce had been diagnosed with or had suffered from a pain disorder associated with a general medical condition and psychological factors at any point prior to the car accident in September 20th, 2003?

A. I didn't find any support for that either.

...

Re Genuineness of Fulcher's Complaints

A. In terms of his mood, from what I gathered is that his depressed mood has actually eased significantly compared to what it had been following the accident, but that he still had many days during the week where he found himself feeling hopeless, feeling depressed. He reported struggles that for me are consistent with difficulties with self-esteem or self-confidence.

...

A. ... In interviewing him also I would -- I would look for ... how forthcoming do I think this patient is being with me; how genuine do I think he's being? And with Mr. Fulcher he wasn't exaggerating things. In fact, he was very much bringing forth things that he talked about as being little improvements here and there with his pain or with his sleep. And he was very open and forthcoming about very difficult issues in terms of his upbringing. So I found in that respect that I didn't have any difficulties engaging him or getting information from him, and I found him to be forthcoming and not manipulative in some way.

[Emphasis added.]

[125] Dr. Levitt gave reasons for limiting his testing to two tests, the PAI and MMPI-2:

A. ... in forensic practice ... there are only two tests that have the research base behind them that -- that I would use in a situation like that. And that's the PAI ... the Personality Assessment Inventory and the MMPI-2 which stands for the Minnesota Multiphasic Personality Inventory.

A. ... And the reason that these two tests in particular are so good for a legal setting is that there's a very strong research base for both the clinical piece of it and the validity scales. And while there's some other tests out there that are good I wouldn't use them in this setting. I would just rely on these two.

...

A. ... And to my knowledge the MMPI-2 is the only one with a scoring program that allows us to compare to people who are involved in a legal action. And that's what this -- that's what these scores are compared to in this profile.

[Emphasis added.]

[126] He said of Fulcher's MMPI-2 Content Scales Profile:

A. Again, this is focusing on different groupings of symptoms so if we look at the high point again the things that would be considered significant compared to a clinical population, compared to patients, the first one we see elevated there is anxiety. And that would be elevated because of both the worries about his future and because of the nightmares. Then depression is also elevated, the DEP. Again, consistent with all that we have noted. The next one elevated, ATA, is health concerns. Again, man with chronic pain that's unrelenting. And then the last one of note, the R -- WRK, that talks about dysfunction at work. He's having difficulty in terms of work, work stress, frustration with work, difficulty of doing work. All of the above could be captured by that. And it would be consistent with what he's talking about, having a really hard time doing work the way he used to do it.

...

[127] Dr Levitt criticized Dr. Travis' conclusions that Fulcher was not genuine and was exaggerating his memory complaints on the basis that Dr. Travis had relied on inappropriate testing. Dr. Levitt gave evidence that the testing used by Dr. Travis was unreliable and invalid in a forensic setting:

Q. Did you review any reports that where an expert came to a different conclusion with respect to Bruce's forthrightness in answering questions?

A. Of all the reports I looked at only one had questions about the -- the validity with the psychological testing. All the rest saw him similarly as I did.

Q. And who was the author of that one report?

A. That was Dr. Travis.

[128] The evidence of Dr. Levitt contains the following:

Q. Right. And in the case of the MMPI-2 which you did for Mr. Fulcher did you utilize a Fake Bad Scale?

A. I did not, no. ... the Fake Bad Scale isn't one that I would rely on in a legal setting. So I don't use it.

[129] He also criticized Dr. Travis' "Fake Bad Scale" scoring:

Q. Did you go back and re score for the Fake Bad Scale?

A. I went back to re score but I don't use it.

Q. What was the result of your Fake Bad Scale rescore?

A. My result was still it did not reach cutoff which means that it's not significant enough to consider it. ...

[130] He continued:

... But the issue is not the score itself. The issue is what the scale -- what the scale's reliability and validity is. The research out there basically shows that at this point it's not best practice to use it in legal settings because it can be elevated when someone has cognitive issues, thinking problems; it can be elevated when someone has pain; it can be elevated when someone has traumatic stress. All those things are things I saw in Mr. Fulcher, so it is hard to interpret a score like that in a setting like this and it's usually wise not use it.

...

Again, the issue with MMPI-2 RF and the reason I only use MMPI-2 and PAI, the [MMP1-] 2 RF is still in the sense of research a work in progress. It is an excellent test, excellent for all sorts of uses clinically, but in a legal setting still the research base is mostly research done by the test publisher. There's not enough broad research done by folks with no vested interest. So the best practice is as I read it in our field for doing legal assessment would be not to use the 2 RF yet at this time that in the sense the jury is still out on that one, so I stay away from it.

[Emphasis added.]

[131] Counsel for the Defendant relied on the evidence of Dr. Travis, who opined, based on the results of the MMPI-2 RF test, that Fulcher was exaggerating memory complaints. He said there were significant elevations on the Response Bias Scale indicating that Fulcher was over-reporting or exaggerating his cognitive complaints:

Q. And what about the Response Bias Inventory?

A. There was a raw score of 13, which would be interpreted ... that exaggerated memory complaints are likely. ... A response by a score of 13 falls within this range of exaggerated memory complaints being likely. ...

[132] Counsel for the Plaintiff cross-examined Dr. Travis on the controversy surrounding the use of the MMPI-2 RF including the Response Bias Scale and the Fake Bad Scale in a forensic context. Dr. Travis' evidence during cross-examination includes the following:

Q. Yes. When was the MMPI-2 RF [Restructured Format] first put into use?

A. In 2008. It's a relatively new instrument in that regard.

A. ... it [the MMPI-2 RF] measures over and above what the MMPI-2 measures.

Q. What are those additional things?

A. The key ones have to do with more sophisticated measures of symptom validity.

Q. All right. Tell us specifically what those are; what they're called?

A. The Response Bias Scale and the FBS scale. The FBS stands for Fake Bad Scale.

...

Q. ... Turning to the test results, you point to a number of results that you say support your concerns about the validity of his responses?

A. That's correct.

Q. Okay. And one of them is the Fake Bad Score?

A. That's correct.

Q. The FBS. You're aware that there's considerable controversy with respect to the use of the Fake Bad Score in a forensic setting?

A. That's correct.

Q. Not everybody accepts that the Fake Bad Score is appropriate to be used in the litigation setting; isn't that right?

A. That's correct.

Q. You didn't tell Her Honour that in your evidence in chief. Would you agree with me that the Fake Bad Score has to be utilized with extreme caution in a forensic setting?

A. Not necessarily at all.

Q. It's not generally accepted in the psychological community, is it?

A. I believe it is.

Q. Are you aware --

A. It is increasingly accepted.

At pp. 123-125

Q. [Quoting from "Best Practices in Forensic Mental Health Assessment Evaluation for Personal Injury Claims," Oxford University Press, 2011] and this text says that: The Fake Bad Scale derived from the MMPI-2 required separate discussion because of the substantial controversy regarding its use. Now, you do understand there's substantial controversy concerning its use, don't you?

A. Yes.

Q. Goes on to say: The arguments have markedly increased since Pearson Assessment have added the FBS to the scores validity on MMPI-2. Now, Pearson Assessment is the company that publishes the MMPI-2, isn't it?

A. That's correct.

Q. They're not a disinterested neutral third party, are they?

A. I would think not.

Q. And it says the FBS includes -- sorry, 43 items from the MMPI-2?

A. Yes.

Q. You'd agree with that, don't you?

A. Yes.

Q. Butcher et al -- and I'll come to the cite in a moment -- question the the validity of the FBS. Their criticisms include (a) actual physical problems caused by high FBS scores.

Does that accord with your impression of what happens with the FBS, people with actual physical problems tend to score high?

A. Not necessarily.

Q. Do you disagree with --

A. Some do.

Q. Okay. And if a person with an actual physical problem scores high that may be an improper representation of what the FBS is supposed to convey?

A. Of course.

Q. Would you agree stress related to litigation may cause elevated scores; do you agree with that?

A. Definitely.

Q. Mr. Fulcher, you've noted, has stress related to litigation, doesn't he?

A. That's correct.

Q. You agree with me that's a factor causing elevated FBS scores for this gentleman?

...

Q. ... This learned book says that in a published article on Butcher et al, which I'll come to in a moment, they noted that stress related to litigation may cause elevated scores, and you'd agree with that, wouldn't you?

A. I guess I would, yes.

Q. And Mr. Fulcher you've noted in your report is stressed by among other things this litigation, right?

A. Yes.

Q. If Mr. Fulcher's FBS score is elevated because of litigation that would result in the FBS not representing what you think it was intended to represent, isn't that true?

A. That's based on an if; it's hard to respond to that.

At p. 126

Q. (E) There are too many false positives.

Now, is that your experience that there can be false positives with the FBS?

A. There can be false positives with nearly every instrument, medical or psychological.

Q. Are there more false positives with the FBS than other generally accepted measures?

A. I don't think so.

Q. So you disagree with the book when it says there are too many false positives.

A. I don't know if there's too many.

...

A. I heard of Butcher. He's a prominent MMP-I researcher who very often disagrees with some of the other people in the field.

At pp. 127-129:

Q. You indicated that Bruce had elevated scores with respect to somatic complaints?

A. Correct.

Q. And those can be indicative of a somatoform disorder?

A. It can.

Q. And that is where someone can manifest emotional distress through physical problems or physical complaints?

A. The other way around, somatic symptoms from emotional problems.

Q. I thought that's what I said, but looking at page 130 of the text one of the things that the authors say is: "People with somatoform disorders who unconsciously present psychological problems via physical symptoms have tendency to score high on the FBS". Is that your experience that people with somatoform disorders tend to score high on the FBS?

A. Not really.

...

Q. And this book refers to at least Florida trial court cases in which the FBS was excluded because it was not generally accepted in the psychological community. Are you aware of any cases where the FBS has been admitted into evidence in Ontario as being generally accepted in the psychological community?

A. No, but I can tell you that a lot of my colleagues who are senior psychologists and neuropsychologists rely on that and, to my knowledge, that has never been shot out of court.

Q. Do you know if it's ever been accepted to -- do you know specifically?

A. I would assume it's been accepted but personally I don't know for sure.

Q. Would you agree that there is a difference between measures that are appropriate in a clinical setting and measures that may be appropriate in a forensic medical legal setting?

A. Yes.

Q. And that it's only the most well researched and documented measures that should be used in a forensic medical legal setting?

A. Yes, bearing in mind that there are contentions of about -- about every psychological instrument that is in common use and there are thousands.

At p. 131

Q. The MMPI-2 RF has been around since I think you said 2008?

A. That's when it was first published.

Q. And it's not yet generally accepted for use in forensic medical legal settings, is it?

A. It seems to me that it is.

Q. Well, let's look at page 134 of this text. The Minnesota Multi-phasic Personality Inventory 2 restructured form contains 338 items from the MMPI-2. In our opinion, this instrument has significant promise and appears likely to improve on the MMPI-2 in some ways, especially because it includes more of the research basis for various interpretations. However, at the present time, we don't believe that the quantity of published peer review research by psychologists, independent of the publisher, Pearson Assessments, is yet sufficient for this test to be utilized in forensic assessments. Do you agree with that statement, doctor?

A. No.

[133] Dr. Travis' cross-examination includes the following:

At p. 132:

Q. So if we leave aside the false -- the Fake Bad Scale, and the MMPI-2 RF, the results beyond those that you relied upon were the MMPI-2 undertaken by Dr. Levitt that you rescored and the PAI; is that correct?

A. Well, there is the Response Bias Scale.

Q. Okay. In terms of the PAI you hand scored it, didn't you?

A. I did.

Q. When you wrote your report setting out the opinions that you've relied upon in this matter, you gave your opinions based on the hand scoring that you had undertaken with respect to the Kaplan and Kaplan testing?

A. Correct.

At p. 133:

Q. And in fact, you made an error hand scoring Mr. Fulcher's PAI, didn't you?

A. I'm not aware of that.

[134] With respect to Dr. Levitt's criticism that Dr. Travis used standard deviations in respect of the general population rather than chronic pain patients, Dr. Travis' cross-examination includes the following:

At p. 138:

Q. And, doctor, would you agree with me that on some of these tests the general population may score differently than say a population of chronic pain patients?

A. Yes.

At p. 139:

Q. And the standard deviations and the figures you gave to Her Honour in your evidence in chief, those were respect to the general population, weren't they?

*A. That's correct.

Q. So I suggest to you, doctor, the fact that Bruce may score high on some of these validity scores has to be measured in relation to other people with chronic pain problems like him, doesn't it?

A. Not necessarily.

At pp. 140-141:

Q. So if you test somebody who's got chronic pain you almost know by definition that they're -- they're going to score high on the SOM scale, aren't they?

A. We expect some elevation, yes.

Q. Some elevation. Don't you have to determine what's clinically significant in that population in reference to standard scores for that population?

A. Not necessarily.

[135] Dr. Travis opined that Fulcher does not have serious and permanent ongoing impairments of important bodily functions. His cross-examination contains the following:

Q. Okay. If one accepts that Bruce Fulcher has the problems with pain, and cognition and insomnia that he described to you, would you agree that he would appear to have a serious and permanent impairment of function?

A. It depends on whether you accept those as bona fide.

Q. I'm saying if you accept they're bona fide, if you accept what he says is true, based on his reports to you?

A. Yes.

Q. Do you agree that he has serious and permanent ongoing impairments of important bodily functions?

A. Not necessarily. I split hairs. He might have serious problems. Whether they're permanent is another issue.

[136] In chief, Dr. Travis questioned Fulcher's commitment to his own rehabilitation. His cross-examination contains the following:

At p. 142:

Q. ... What exactly is it that you think that Bruce Fulcher should be doing that he's not, Dr. Travis?

A. Starting with sleep. Good sleep hygiene is conducive to getting good sleep, and it's conducive to controlling pain to a very large extent.

Q. What is it that you say that Bruce Fulcher is doing that has actively contributed to poor sleep hygiene?

A. Sometimes he goes to bed; sometimes he doesn't go to bed at all.

At p. 143:

Q. Okay. So other than that Bruce should sleep better, what else do you say that Bruce Fulcher should be doing to promote a healthy life style and resumption of normal life activities that he's not presently doing?

A. To some extent, he's made changes already. He's now more physically active; and he is working out and he is doing yoga. He has lost considerable weight. These are -- these are steps to the -- the good.

Q. Well, he had done all of those things before you authored your report, hadn't he?

A. That's correct, those things.

At p. 144:

A. ... I didn't get good factual information on exactly what is his work routine; what are his hours of work; when does he go in; what does he do to control his pain, regulate his pain throughout the day. These are things that regulation of those things by routine is a good way of controlling pain and improving performance on jobs.

Q. But you said you didn't get a good history from him about those things, so how do you know he's not doing them?

A. He didn't provide a good history on those things.

Q. Did you ask for collateral sources of information to satisfy yourself about whether in fact?

A. I have already told you I haven't, nor did I have time.

[137] Dr. Travis concluded Fulcher is not vocationally impaired or disabled by cognitive factors. His cross-examination includes the following:

At p. 145:

Q. You say in your report: There is no bona fide evidence that Mr. Fulcher has been rendered vocationally impaired or disabled by cognitive factors. That's your opinion?

A. That's correct.

...

At p. 148:

Q. And, doctor, are memory and concentration problems -- problems that individuals frequently have when they're chronically sleep deprived?

A. Yes.

Q. And if a person has difficulties with reading comprehension could that put them in a vocationally impaired category?

A. It could.

[Emphasis added.]

[138] In concluding Fulcher was not being genuine, Dr. Travis referred to Fulcher's tendency to "split hairs:"

Q. What do you mean splitting hairs?

A. Constantly asking what does this mean and appearing to have difficulty responding to the questionnaires themselves. In effect, constantly coming back to me to redefine it and so forth.

[139] In his evidence, Dr. Levitt referred to Fulcher's tendency to "split hairs" in relation to his scores on "inconsistency," as follows.

A. ... Typically, when someone has a higher score on inconsistency it's because they're confused and have reading problems or because they're not concentrating.

In Mr. Fulcher's case, I saw something very different actually. This is a man who -- and I have also seen this in another report in the file -- who splits hairs. The exact meaning of things is very important to him. And he seemed to put a great deal of effort into making sure he was getting across exactly what he was trying to get across. So questions that for most people would look pretty much the same; for Mr. Fulcher, he looked at them and the slight difference in meaning was important enough for him to answer them differently. It wasn't that he was trying to be misleading or that he didn't understand things is that he's from, what I could gather, really trying very hard to be as honest about each item, so I think he took a very painstaking approach to it.

...

[Emphasis added.]

[140] Dr. Travis' evidence with respect to causation of an adjustment disorder was as follows:

The Witness: ... the basis for any such adjustment disorder is obviously multi-factoral. One cannot infer that this condition was caused by or is fundamentally attributed to the injuries sustained as a result of the subject motor vehicle accident. And here I'm stating that there's a whole lot of things and a whole lot of factors that have come into play that go beyond the circumstances of the actual accident itself.

[141] As to a pain disorder he said:

One must consider the presumptive diagnosis of a pain disorder associated with both psychological factors and a general medical condition. Regardless, the basis for inferring that any such pain disorder is psychologically or vocationally disabling is lacking. ... there's considerable disagreement on the -- among the medical professionals, particularly orthopaedic surgeons, as to the -- the onset of symptoms, the attribution of symptoms to the accident as opposed to other factors. ... But those are medical issues. I have stated that Mr. Fulcher has good reason to be depressed and dissatisfied given his ongoing pain and pain related physical limitations both before and after the subject accident and the number of situational stressors and life changes that have been in place since the subject accident.

Dr. Travis' Diagnosis

[142] Dr. Travis' diagnosis was as follows:

At p. 94:

Q. ... what is your own opinion with respect to Mr. Fulcher's psychological status?

A. My diagnostic formulation is contained on pages 43 and 44. What I have said is that Mr. Fulcher's poly-symptomatic presentation and the evidence of over-reporting of symptoms on the psychological inventories complicated the process of formulating an accurate differential diagnosis. Because of that, the differential diagnostic process must be limited to offering a presumptive diagnostic formulation, one, that is ... based upon a reasonable appreciation of the circumstantial information at hand, rather, than one founded upon solid confirmatory empirical evidence. What I have said is that ... in my view, no behavioural evidence of any major or disabling psychiatric disorder.

At pp. 95-96

Q. ... with respect to Axis 1 your conclusions are what?

A. First of all, there's no behavioural evidence of any major or disabling psychiatric disorder. He doesn't conduct himself as though he's a ... psychiatrically disabled person. ... there's no evidence of shortage of -- or loss of intellectual horsepower or intellectual deficit, at least at this level of assessment. What I've stated is that chronic pain has been the primary ongoing issue before and after the subject accident.

At pp. 97-98

A.... He's got good reason to be depressed. I said given the number of life changes and adjustment challenges associated with them it is reasonable to consider the presumptive diagnosis of adjustment disorder as an Axis 1 disorder. However, the behavioural presentation and responses to the Beck measures of depression and anxiety indicate that he -- indicated remarkably mild symptoms of depression and anxiety. Other instruments suggest that he is more depressed than these Beck instruments suggest.

...

At p. 101:

Q. Okay. And you made some comments with respect to **Axis 3**?

A. Yes, basically, I have said these are medical issues. They pertain to general medical conditions. Those are for the physician's, medical specialists, to deal with. I did note that there were again significantly differing medical surgical opinions on the issues of attribution and expected life consequences.

Q. With respect to **Axis 4**?

A. I thought these were significant. They included -- the list includes change in occupational duties, reported hours of work and remuneration, dissatisfaction with career situation, uncertainty about future career and earnings prospects, changes in physical and recreational activities, poor sleep hygiene.

At p. 102:

A. ... the lack of consensus as to the nature or severity in expected life consequences of his accident related injuries and the demands of the ongoing medical legal and case building processes are also highly significant as perpetuating factors, psychosocial and environmental factors. Now for the same reason one can't make a positive psychological diagnosis, that is a clinical -- diagnosis of clinical disorder, it's very difficult to provide a reliable global assessment of functioning.

At p. 103:

Q. ... your findings with respect to Axis 5 then?

A. Difficult to estimate.

At p. 104:

A. ... From the cognitive and psychological perspectives, there's no basis for concluding that Mr. Fulcher has been rendered unable to carry on with normal life activities as a result of his ... current accident related injury.

Conclusions re Disputed Facts re Causation

General Observations re Medical Evidence on Cause of Fulcher's Physical Impairments

[143] On the matter of causation, I have considered the evidence of Drs. Paitich, Newell, Clements, Shiffer and Mailis.

[144] Dr. Paitich's evidence differed in important respects from the evidence of the other medical witnesses.

[145] Dr. Paitich's evidence that he found no evidence of spasm (an objective finding) was inconsistent with the findings of Dr. Newell, also an orthopaedic surgeon, and Drs. Green, Mailis, and Clement, who all observed and noted spasm. Dr. Paitich conceded in cross-examination that if other orthopaedic surgeons found spasms, he would accept that finding.

[146] Dr. Paitich [who is not a neurologist] based his opinion in part on no change in Fulcher's neurological condition between July 2003 and January 2004. Dr. Paitich said he did not know whether Dr. Shiffer had concluded that Fulcher's neurological condition had deteriorated after the accident. He seems to have assumed that Fulcher's level of neurological functioning before and after the accident was about the same.

[147] Counsel for the Defendant emphasized Dr. Paitich's evidence to the effect that the mere fact that Fulcher had complaints of increased back pain didn't necessarily translate into neurological findings.

[148] I have quoted the evidence of Dr. Shiffer [who is a neurologist] that his neurological condition had in fact deteriorated.

[149] Dr. Paitich assumed that in July 2003, Dr. Shiffer found pain of sufficient severity to warrant the ordering of a CT scan. I have already noted that it was not Dr. Shiffer who ordered the CT scan, but Dr. Green. [He had ordered it in May of 2003, at a time when Fulcher said he was having more severe problems than he was reporting to Dr. Shiffer in July.]

[150] During his cross-examination, Dr. Paitich and counsel for the Plaintiff engaged in some verbal sparring about "sciatica." Used in its technical sense, Dr. Paitich acknowledged that "sciatica" is a neurological finding, not just a finding of pain. He agreed that if Fulcher had true sciatica [involving nerve compression] after the martial arts incident, that had largely resolved before the accident but returned after, that would be important.

[151] Dr. Paitich did not bring his whole file with him when he attended to give evidence at trial. He didn't bring the binders [including the radiological brief, a neurological report of Dr. Shiffer, the clinical and operative reports of Dr. Schutz, the report of Dr. Clements he reviewed before he prepared his opinion. Prior to giving evidence, he did not review much of the documentation that had provided the basis for his conclusions. In cross-examination, he

said that he reviewed only his own Report and Addendum Report, the MRI and the CT scan reports.

[152] Dr. Paitich did not personally review the image of the May 30, 2003 CT scan. He didn't review the digital images of the MRI. He agreed a CT scan and MRI scan are not directly comparable.

[153] In reaching his conclusions on causation, Dr. Paitich said he had no information about the speed of the Conklin and Fulcher vehicles at impact [Conklin 35-40 kpm; Fulcher stopped] or the damage to the Conklin vehicle [\$13,663.66.]

[154] Although most of the other medical witnesses including Dr. Travis noted that Fulcher demonstrated observable pain behaviours during their examinations and stood during portions of the examination, Dr. Paitich maintained that he sat for a full 50 minutes without displaying any evidence of sitting intolerance.

Conclusions re Causation of Fulcher's Ongoing Physical and Cognitive Impairments

1(a) Could a rear end collision cause or aggravate a disc herniation?

[155] I find the answer is yes. I have noted that Dr. Paitich agreed in cross-examination that whiplash injuries are generally characterized as flexion/extension injuries. I find that Fulcher's lumbar spine likely extended, then flexed.

[156] I note Dr. Paitich's evidence on cross-examination, reject his evidence in chief and accept Dr. Newell's evidence in this regard. I find this rear-end collision did cause and aggravate Fulcher's pre-existing disc herniation.

1(b)(i) Would Fulcher have needed surgery in January had the Accident not happened?

[157] I prefer the neurological evidence of Dr. Shiffer and the evidence of Drs. Mailis and Newell (like Dr. Paitich, an orthopaedic specialist) that Fulcher had a vulnerable back and that the accident materially contributed to the worsening of his symptoms, to the evidence of Dr. Paitich. I reject Dr. Paitich's evidence that the accident did not cause Fulcher's ongoing back pain. Dr. Newell noted Fulcher's improvement in the interim between July 2003 and September 2003. Dr. Paitich knew little about that. While Dr. Paitich emphasized that Fulcher's July 2003 condition warranted surgery, Dr. Shiffer obviously disagreed, as she did not refer him to Dr. Shutz in July. Dr. Shiffer gave evidence that in July 2003, she recommended conservative treatment because she believed his injuries could resolve without surgery.

[158] Dr. Paitich assumed Fulcher's neurological condition did not change between July 2003 and October 2003. On neurological matters, I prefer the evidence of Dr. Shiffer, a neurologist, to the evidence of Dr. Paitich, an orthopaedic surgeon. I accept Dr. Shiffer's evidence that Fulcher's neurological signs were worse in October [after the accident] than they had been in July 2003.

[159] I accept Fulcher's evidence that his pain was much worse immediately after the accident. I reject Dr. Paitich's evidence that surgery would have been necessary in January

2004 whether or not the accident had occurred. Dr. Paitich had little information about the relative severity of Fulcher's complaints and symptoms after the martial arts injury in November of 2002 and after the accident. I prefer the evidence of Drs. Mailis, Newell, Clements and Shiffer that the accident precipitated the surgery.

[160] I accept Dr. Shiffer's evidence that Fulcher was more vulnerable to further bulging or herniation, and that worsening of pain could indicate "movement of disc material a couple of millimetres may be all it takes to have intractable discomfort."

[161] The answer is no, Fulcher would not have needed surgery in January 2004 but for the accident.

1(b)(ii) But for the Accident, When would Surgery have been Necessary?

[162] I accept the evidence of Dr. Newell that surgery would have eventually been necessary. He said maybe in 5 or 10 or 15 years, then settled on 20 years from November 2002. I shall refer to this evidence again when assessing Fulcher's damages.

1(c) Did the Accident Cause Fulcher's Ongoing Physical Impairments?

[163] I accept Dr. Shiffer's, Fulcher's Austin's and Pauline's evidence that between July 2003 and until September 2003, his condition was not terribly serious.

[164] I accept Fulcher's evidence about his ongoing physical disabilities at the time of trial. I find based in part on Dr. Newell's evidence, that the accident caused the surgery and the surgery caused his ongoing physical impairments/segmental instability, which in turn caused chronic unresolving back pain. I find on the evidence that his disabilities are unlikely to improve.

[165] I accept the evidence of Drs Shiffer, Mailis and Newell that Fulcher's physical complaints, including stiffness, inability to bend, stretch and lift, are genuine.

[166] Dr. Paitich conceded it would be important if others observed spasms, objective symptoms. Drs. Green and Mailis observed the spasms. He did not. I reject Dr. Paitich's opinion that Fulcher was "faking."

[167] I accept the evidence of Dr. Mailis and Dr. Shiffer that the accident materially contributed to the worsening of his symptoms. I accept the evidence of Drs. Mailis and Shiffer. The accident caused the surgery. The surgery caused the ongoing physical problems. Therefore, the accident caused the ongoing physical problems.

1(d) Did Fulcher's Accident Cause Ongoing Cognitive Impairments?

[168] I generally prefer the evidence of Drs Levitt and Kaplan to the evidence of Dr. Travis. Dr. Travis used and relied on MMPI 2 RF, knowing its use is controversial in forensic settings without advising this Court of that controversy. I accept the evidence of Dr. Levitt with respect to the proper use to be made of the MMP-2RF, the Fake Bad Scale and response bias scale. If used at all in forensic settings, they should be used with caution. I also accept Dr. Levitt's criticism of Dr. Travis that in reaching his conclusions, Dr. Travis used

standardized deviations applicable to the general population, not to the chronic pain population.

[169] Overall, Dr. Travis presented as an advocate for the defence rather than as an independent objective witness. I have included a number of quotes from his evidence.

[170] I reject Dr. Travis' conclusion that Fulcher was exaggerating his cognitive impairments. I accept Fulcher's evidence that pain and lack of sleep since the accident have caused his mind to be "fuzzy, like mud." I have accepted that his pain, insomnia and cognitive deficits are real. I accept Dr. Levitt's opinion that Fulcher's cognitive impairments are likely to be permanent.

[171] I find that Fulcher's genuine physical and cognitive difficulties have combined in the past and continue to combine to compromise his ability to promote and sell Novitherm's products, to perform physical tasks such as inspecting equipment and measuring buildings and cognitive tasks such as preparing reports and proposals.

[172] I accept the evidence of Drs Mailis and Drs. Levitt and Kaplan that Fulcher's physical and cognitive problems affect his ability to gain and service customers and to prepare accurate case studies and proposals and to measure buildings, assess and adjust furnaces, boilers, convectors. He needs to bend, twist and lift when he engages in these physical activities and to concentrate and focus when preparing studies and proposals. While his impairments may diminish to some extent in the future, I find they are serious permanent impairments of important physical and psychological functions.

Disputed Facts re Quantum of Fulcher's Past and Future Loss of Income/Earning Capacity

(a) What Profits did Novitherm Lose as a Result of Fulcher's Injuries?

[173] Counsel for the Plaintiffs submitted Fulcher's injuries and disabilities have adversely affected Novitherm's and in turn Fulcher's earnings, both in respect of his compensation for services he has provided to Novitherm and in respect of his ownership interest.

[174] I have already found that Fulcher's physical and cognitive problems have compromised and continue to compromise his ability to examine and adjust heating equipment, to prepare reports, case studies and proposals, and to sell Novitherm products. He has been unable to develop the GTA market as he did before the accident or to develop the Ottawa market at all. Snyder has found it necessary to do much of the work Fulcher did before the accident, taking Snyder away from developing the Montreal market. I accept that the overall result has caused Novitherm to generate less revenue than it would have generated had Fulcher not been injured.

[175] The real issue is the extent of Novitherm's loss and Fulcher's damages flowing from it.

[176] Counsel for the Plaintiff called Mr. McCauley ("McCauley"), a specialist in forensic accounting and a business valuator, to calculate Fulcher's past and future loss of earnings/earning capacity, having reference to Novitherm's performance and financial records before and after the accident.

[177] McCauley gave evidence before I struck the jury. He projected Novitherm's financial results as if the accident had not happened, compared those projections to its actual results post-accident and then calculated Novitherm's and Fulcher's losses as a result of the accident.

[178] He noted that before the accident, Novitherm's revenues had jumped from \$47,649 in 2000, to \$147,025 in 2001, \$457,460 in 2002, and \$990,683 in 2003 [the last full year before the accident.]

[179] Had the accident not occurred and Fulcher not been injured, he assumed that Novitherm's revenues would have increased by 35% in each of 2004 and 2005; 20% in 2006 and 10% in each of 2007, 2008 and 2009. In 2010 and 2011, he assumed a 10% decrease in revenues. In each of 2012 and 2013 he assumed 10% growth.

[180] He assumed that if the accident had not occurred, Novitherm's future gross profit and sales in each fiscal year would have borne the same relationship to each other as they actually have been to each other since the accident.

[181] He assumed Novitherm's business expenses would increase at the same rate as its sales have actually increased, or would decrease at the same rate as its sales have actually decreased since the accident. However, in each of 2004, 2005 and 2006, he assumed that Novitherm's expenses would have increased by an additional \$100,000 [over and above those assumed expenses.]

[182] McCauley's revenue assumptions for Novitherm as compared to its actual revenues since the accident are set out in Chart 1:

CHART 1		
Year	Actual Revenue Since the Accident	McCauley's Assumed Revenue but for the Accident
2004	\$895,006	\$1,486,029
2005	\$708,245	\$2,006,139
2006	1,276,689	\$2,407,367
2007	\$690,349	\$2,648,104
2008	\$1,380,434	\$2,912,914
2009	\$1,611,211	\$3,204,205
2010	\$1,510,549	\$3,043,995
2011	\$1,082,104	\$2,739,596
2012		\$3,013,556

[183] Counsel for the Defendant did not call any evidence to assist the Court in projecting Novitherm's profits or losses but for the accident. He called Mr. Hoare ("Hoare"), who although qualified to do so, did not attempt to predict the performance of Novitherm had the accident not happened or to calculate Fulcher's losses as a result.

[184] Hoare did not use Novitherm data at all in estimating and calculating Fulcher's compensation loss. On the instructions of counsel for the Defendant, he used statistical averages for marketing sales managers with university degrees.

[185] Although his estimates of gross annual income were derived from statistics not derived from Novitherm data, he deducted from his statistical averages amounts that Fulcher actually received from Novitherm to calculate Fulcher's losses.

[186] Using the statistical averages and deducting Fulcher's actual compensation earnings from Novitherm in each year, Hoare calculated 80% of net of Fulcher's pre-trial compensation/earnings, without reference to dividends, at \$274,400.

[187] Hoare did criticize McCauley's projections and methodology.

[188] Hoare criticized McCauley's assumptions about Fulcher's compensation income. He opined that McCauley's assumed increases were well above what employees of Fulcher's background, history, education, etc. would normally receive.

[189] Hoare opined that McCauley's projections of revenue but for the accident were too high and his assumptions of Novitherm expenses were too low. Expenses necessary to achieve his projected rate of growth [e.g., expenses for increases in sales staff, etc.], would have exceeded the \$100,000 increases McCauley assumed in 2004, 2005 and 2006.

[190] Hoare criticized McCauley's assumption that all decreases in Novitherm's revenues after the accident were attributable to Fulcher's injuries. He opined that McCauley should have recognized other negative factors that have affected Novitherm's business since September 2003.

(b) What Would Fulcher's Performance-Based Compensation from Novitherm Have Been but for the Accident?

[191] McCauley's estimates of Fulcher's compensation based on Novitherm records, adjusted to coincide with the December 31 year end, and Hoare's based on statistical averages paid for services performed each calendar year but for the accident were as set out in Chart 2 below:

CHART 2		
Year	Hoare	McCauley
2004	\$108,481	\$110,000
2005	\$111,085	\$132,000
2006	\$113,085	\$151,200
2007	\$116,364	\$166,320
2008	\$118,691	\$182,952
2009	\$120,234	\$186,873
2010	\$122,639	\$172,977
2011	\$124,969	\$172,067
2012	\$124,969	\$189,273

[192] McCauley's estimate of Fulcher's compensation income from Novitherm but for the accident was based on the assumption that Fulcher's compensation income for services performed for Novitherm would have increased to \$100,000 in the year ended June 30, 2004 and increased by 20% per year so long as the actual increase in Novitherm's sales was at least 20% year over year. [If it were less, he would limit Fulcher's compensation income to increase by that lower percentage.] Because he assumed increases in Novitherm's revenues of at least 20% per year except in 2010 and 2011, Fulcher's assumed compensation income increased by 20% per year except in 2010 and 2011.

What would Fulcher's Ownership Income from Novitherm have been but for the Accident?

[193] Hoare made no attempt to estimate Fulcher's losses in connection with his ownership interest in Novitherm.

McCauley's Evidence

[194] McCauley attempted to quantify Fulcher's loss as an owner of Novitherm as a result of the accident. He assumed that 35% of Novitherm's net income would have been available for distribution 65% to Snyder and 35% to Fulcher in each year post-accident.

[195] Based on his review of Novitherm records and the assumptions already set out, McCauley calculated Fulcher's ownership losses [in addition to his compensation losses shown in Chart 2] as set out in Chart 3 below:

Chart 3	
Year	Fulcher's Share of Net Income (Loss)
2004	\$(11,917)
2005	(51,271)
2006	47,844
2007	166,815
2008	164,176
2009	93,464
2010	104,651
2011	148,271

Fulcher's Total Income Loss/Loss of Earning Capacity

[196] After calculating Fulcher's compensation and ownership interest, to calculate Fulcher's total income loss, McCauley deducted amounts Fulcher had actually received for services performed over the same time period.

[197] On Exhibit 35, McCauley calculated Fulcher's total gross income including compensation from his ownership interest as set out in Chart 4 below:

Chart 4		
Year	Compensation + Share of Novitherm Net Income before Income Taxes – June 30 – June 30	Compensation + Share of Novitherm Net Income before Income Taxes on a Calendar basis
June 30/03-June 30/04	\$112,612	\$98,083
June 30/04-June 30/05	85,554	80,729
June 30/05-June 30/06	77,904	199,044
June 30/06-June 30/07	320,184	333,135
June 30/07-June 30/08	346,085	347,128
June 30/08-June 30/09	348,170	280,337
June 30/09-June 30/10	212,503	277,628
June 30/10-June 30/11	342,752	320,338
June 30/11-June 30/12	297,922	n/a
June 30/12-June 30/13	327,714	n/a

[198] When the statutory formula [80% of net] was used, McCauley calculated Fulcher's past earnings of \$736,779, rounded to \$736,800. After a 12% deduction for business uncertainties, 80% of net was \$709,895.

[199] As noted earlier, using the statistical averages and deducting Fulcher's actual compensation earnings from Novitherm in each year and without including anything for Fulcher's losses in connection with his 35% ownership interest, Hoare calculated 80% of net of Fulcher's pre-trial compensation/earnings, without reference to dividends, at \$274,400.

Future Loss of Earnings/Earning Capacity

McCauley's Evidence

[200] To calculate Fulcher's future loss of earnings/earning capacity, McCauley used the same assumptions and methodology he used to calculate pre-trial losses. However, he assumed for the year ended 2011 that his post-accident compensation income would be \$99,000, in 2012 and 2013 \$98,000 and that after 2013, it would increase annually at the rate of inflation. He assumed after 2013, Novitherm's revenue would increase at the rate of inflation. He used a 12% discount to reflect business risk.

[201] On that basis, McCauley calculated Fulcher's loss of future income from December 2011 to age 60 at \$866,700 and to age 65 at \$1,253,300.

[202] Therefore, he calculated Fulcher's total loss of income capacity, past and future as of December 2011 to age 60 at \$709,895 + \$866,700 to age 65 at \$709,895 + \$1,253,300 = \$1,963,195.

Hoare's Evidence

[203] Hoare produced Exhibit 54, allowing for calculation of future losses by providing factors necessary to generate \$1000 per year. In addition to the statutory present value factor, it includes a mortality factor and employment rate factor, yielding a combined factor and cumulative combined present value factor.

Conclusions re Disputed Facts re Lost Earnings/Lost Profits/Loss of Earning Capacity

A. General Observations

[204] In this case, counsel for the Defendant served a jury notice. Early in the trial, counsel for the Plaintiff moved to strike the jury notice on the grounds of complexity. I refused to do so at that time. I expressed concerns about complexity and exhorted counsel to help find ways to simplify the evidence and to make it as straightforward as possible for the jury to assess damages. A jury was chosen. After McCauley gave evidence and counsel for the Plaintiff renewed his application to strike the jury, for reasons contained in the Trial Record, I ordered the jury to be struck.

[205] Since *Teno, Thornton, Andrews and Lewis v. Todd*, the Supreme Court of Canada has made it clear that pecuniary losses are to be arithmetically calculated having regard to the evidence in each case. Given my conclusion that Fulcher's income but for the accident would have been tied to Novitherm's results and the resulting need for the fact finder to review and scrutinize the financial records of Novitherm, the state of the Novitherm records, the different but complex methodologies of the experts, the numerous assumptions and calculations of McCauley and to determine the reasonableness of McCauley's assumptions and of Fulcher's and Snyder's optimistic projections and ultimately to calculate Fulcher's loss, I concluded it would be unrealistic to expect the jury to review and analyze this evidence and then use it to mathematically calculate Fulcher's damages.

[206] Having now considered the matter in detail, I am even more strongly of the view the matter was too complex for a jury and that leaving this matter with the jury would have been unwise.

B. Factors Complicating the Assessment of Lost Earnings/Lost Earning Capacity

[207] This Court has had difficulty assessing Fulcher's past and future lost earnings/earning capacity.

[208] My difficulty stemmed from a combination of factors:

1. The Sorry State of the Novitherm Financial Records

[209] One counsel described the Novitherm records as "a mess." Nevertheless, McCauley used them as the basis for his projections and I have concluded they must be considered in determining Fulcher's loss. For reasons set out below, they provided a less than totally reliable basis on which to project Novitherm's lost revenues and income, and in turn on which to predict and assess Fulcher's losses.

2. The Novitherm Year End

[210] Novitherm has a June 30 year end.

[211] To use its results and compare them with other information presented for each calendar year, it was necessary to recalculate the Novitherm results as of June 30 in each year to reflect its results for each calendar year. For example, Ex. 35 using data from Novitherm's financials for the year ended June 30, shows Fulcher received \$20,250 compensation for the year ended June 30, 2008 and \$70,000 for the year ended June 30, 2009. To calculate his 2008 calendar income, it was necessary to add \$20,250 and \$70,000 and divide by two = \$45,125 income in the calendar year 2008.

3. Unclear Methodology for Splitting Novitherm Profits

[212] Fulcher and Snyder had no agreed-upon methodology to determine annual compensation or to divvy up Novitherm profits. They had no written shareholders' agreement.

4. Dearth of Evidence linking Novitherm's Post-accident Losses and Fulcher's Compromised Capacity

[213] While Fulcher and Snyder gave evidence, for instance, that after the accident they had to forego developing the Ottawa and Montreal markets as planned, there was little specific evidence about the business they intended to develop before the accident, the pre-accident steps taken and the post-accident effects. There was no specific evidence for instance about specific Ottawa buildings Fulcher tested or measured, or intended to test or measure, before the accident. There were no records tracking or recording Fulcher's or Snyder's pre- and post-accident activities and sales or attributing particular pre- and post-accident sales to either.

5. Other Factors Affecting Novitherm's Results Post-Accident

[214] Counsel for the Defendant submitted it would be an error to assume, as McCauley did, that Fulcher's injuries were the only negative factor affecting Novitherm's actual performance after September 20, 2003. On the evidence, it would be reasonable to conclude that for reasons other than the accident, Fulcher and Snyder have not been able to grow the Novitherm business to the extent they wanted or expected. Since the accident, Novitherm has not made the investments necessary to increase its capacity to expand its markets and revenues. A comparison of Exhibit 20, an organizational chart from 2007, Exhibit 21, a recent print-out from the Novitherm website, suggests that Novitherm has virtually the same personnel today as it had in 2007.

[215] There was evidence of a decrease in natural gas prices that would have reduced the savings generated by Novitherm's heat reflectors and made them more difficult to sell.

[216] An Enbridge Gas incentive programme, in effect before Fulcher's accident, ended after it.

6. Fulcher's Evidence on Financial Matters

[217] Fulcher had little grasp of Novitherm's finances. Although he has signing authority, he has never signed any Novitherm cheques. He did not even know when his Novitherm shares were transferred to him. Fulcher's evidence on financial matters was of little assistance to this Court. Fulcher's company charges Novitherm for Fulcher's services but Fulcher does not decide how much his company will charge.

7. Snyder's Evidence about Novitherm's Finances

[218] Although Snyder and Fulcher share in decision-making, it appears that Snyder does most of the decision-making on financial matters at Novitherm.

[219] Most of the financial information pertinent to the determination of damages came from Snyder.

[220] Snyder gave evidence that someone at Novitherm prepares invoices for Fulcher's services on behalf of Fulcher's company 1644400 Ontario Inc. Novitherm pays the amounts shown on those invoices to Fulcher's company. While Snyder consults with Fulcher, Snyder decides how much compensation Fulcher/Fulcher's company will receive.

[221] It appears that Novitherm's accountant takes instructions from Snyder. McCauley's evidence on Fulcher's losses in relation to his ownership interest was calculated based on 35% of amounts available for payment to shareholders after payment of expenses. On several occasions, Novitherm's accountant has made adjustments to the Novitherm general ledger to reflect amounts that Snyder has directed to be paid out of Novitherm to other companies he personally owned for reasons unrelated to Novitherm business. [In 2006, for instance, Snyder caused Novitherm to pay \$181,000 to a company related to Snyder but unrelated to Fulcher.] In cross-examination, Snyder agreed he was creating expenses in Novitherm that would reduce the amount available for notional distribution to the shareholders (for example, on the last line of Sch. 7 to Ex. 35.)

[222] When I reviewed for example, Exhibit 35, I questioned whether all payouts to Snyder had been included in management fees, or whether some payments to him had been included in professional fees. This in turn caused me to question the accuracy of amounts shown as net income prior to payment to owners.

C. This Court's Approach to the Assessment of Fulcher's Damages for Loss of Earnings/Earning Capacity

[223] I made the factual findings about causation and the extent and duration of Fulcher's impairments before I attempted to quantify his loss of earnings/earning capacity based on those findings.

[224] Because I concluded that my findings on Fulcher's lost earnings/earning capacity should be based at least in part on Novitherm's past and projected earnings, I pored over the available financial records despite the reservations mentioned above.

[225] I attempted to understand Fulcher's vague evidence on financial matters and Snyder's evidence, including his evidence about the Novitherm decision making process, how management determined income for services and categorized it on the financial statements. I also attempted to gauge the extent to which Snyder's year end decisions to use income from Novitherm to gain tax benefits unrelated to Novitherm affected the accuracy of McCauley's conclusions on Fulcher's ownership losses, if at all.

[226] I scrutinized the evidence used and projections/calculations made by the two expert forensic accountants and considered their very different methodologies in arriving at very different conclusions.

General Comments on McCauley's and Hoare's Evidence

[227] I concluded that I could not adopt all of either McCauley's or Hoare's conclusions.

McCauley's Methodology and Approach

[228] McCauley's calculation of Fulcher's lost income (compensation and ownership income) rested on his assumptions about Novitherm's revenue growth and expenses but for the accident. In my view, they were overly generous to Novitherm and failed to reflect other factors apart from Fulcher's injury that have negatively affected the Novitherm business.

[229] In my view, Fulcher's injuries were not the only adverse factor affecting Novitherm's bottom line post-accident. Other factors negatively affecting Novitherm revenues and profits included the drop in natural gas prices since the accident, the end of the Enbridge incentive programme and Novitherm's failure to increase the size of its workforce to foster growth.

[230] Since I have concluded that McCauley's calculations of income share were overly optimistic, it follows that his estimates of lost income due to Fulcher's injuries were too high.

Hoare's Methodology and Approach

[231] Hoare's estimates did not tie Fulcher's compensation income to his reasonable expectations of compensation as an owner of Novitherm. They in no way reflected the reality that if profits warrant, owners of successful businesses are better compensated than employees.

[232] Hoare's calculations of Fulcher's losses did not reflect any of Fulcher's past or projected losses as a 35% owner of Novitherm.

[233] Hoare's methodology mixed apples and oranges. While he ignored the upside of ownership of Novitherm in estimating income in calculating Fulcher's loss, he deducted the benefits Fulcher has received from Novitherm.

Overall Approach to the Calculation

[234] This Court must do the best it can with the evidence it has: *Mulroy v. Aqua Scene*, [1982] O.J. No. 87 (C.A.) In making these observations, I am not being critical of counsel, who were both of great assistance.

[235] In considering past earnings, I was of the view that the amounts Fulcher drew from the company between 1996 and 2002 should not be seen as indicative of the true value of the services he provided to Novitherm during those years. During those years, Fulcher forewent a market salary so that in the future as an owner, he would benefit from the upside of the growth in the business and the likely increase in remuneration over and above statistical averages that likely would have occurred had the momentum in the business been maintained and his abilities not been compromised.

[236] In my view, both the upside and downside of ownership must be reflected in this damages calculation.

[237] I have found on the facts that Fulcher's physical and cognitive deficits resulting from the accident are continuing to cause losses. I have rejected the submissions of counsel for the Defendant that at present Fulcher is not suffering from any loss of any income.

Past Loss of Earnings/Earning Capacity

[238] While I have refused to accept the conclusions of either McCauley or Hoare *holus bolus*, they have been helpful in establishing the outer limits of the spectrum of damages for 80% net past loss earnings.

[239] I have considered and rejected the submission of counsel for the Defendant in argument to the effect that Hoare's estimate of income loss was too high. I have compared Hoare's \$274,700 for past loss (based on statistical averages for employment income unrelated to Novitherm and not including anything for loss of Fulcher's ownership interest) and McCauley's estimate of \$709,895 for past loss.

[240] Having rejected McCauley's assumptions as too generous and Hoare's approach as non-reflective of the Novitherm reality, I have considered a number of possible other approaches to the assessment of Fulcher's losses of income/income capacity.

[241] Counsel for the Plaintiffs suggested in argument that damages could be calculated based on 35% of Novitherm's projected net earnings before any distribution to the owners for fees or management services. To derive Fulcher's loss, I could find that Fulcher's contributions to Novitherm have been compromised by a specific percentage and apply that percentage to the 35% of projected earnings figure.

[242] Using McCauley's projections of lost earnings to date of trial, counsel for the Plaintiff roughly calculated that 80% of net reflecting 25% of 35% of Novitherm's projected net earnings would be \$276,653.88; 50% would be \$553,307.76; 75% would be \$829,961.64.

[243] I note that there was some evidence upon which percentage compromise conclusions could be reached. Fulcher and Snyder gave evidence and Dr. Green's notes dated January 19,

2004 suggest that Fulcher suffered at least a 50% loss of capability to perform his tasks at Novitherm between September 20, 2003 and January 19, 2004; a 100% loss from January 27, 2004 to April 2004; a 75% loss after April 2004 as per Snyder's letter already quoted. In 2005, Fulcher reported a 50% improvement in his pain levels since the surgery to Dr. Mailis-Gagnon. At trial, Fulcher said he has 1 or 2 good days a week, 1 to 2 bad days per week with the balance being somewhere in between. He estimated he was 20-25% as productive at Novitherm as he was pre-accident. Snyder said Fulcher was working at about 50% of his pre-accident capacity.

[244] While I did not use this approach because I have held McCauley's numbers to be too high, the numbers derived from it were of some assistance in arriving at my final conclusions.

[245] In considering past loss of compensation for services, I have borne in mind that Fulcher has actually received, on average, \$58,128 per year since the accident. But for the accident, had the business been successful, Fulcher and Snyder would reasonably have expected to receive higher compensation for services performed than non-owner employees would have received.

[246] I have considered that Fulcher has lost ownership income from Novitherm and that that loss should be reflected in my assessment. However, I have not adopted McCauley's suggestion to add 35% of profits available to owners in each year. I have concluded it was unlikely that Fulcher would have received 35% of the profits of Novitherm in each year.

[247] I have considered, among other things, Hoare's and McCauley's overall assumptions, as well as the compensation Fulcher actually received (adjusted to reflect amounts received in each calendar year), summarized in Chart 5 below:

Chart 5					
	1	2a	2b	2c	3
Year	Hoare's Assumptions on Compensation Income based on statistical evidence for employees and including zero to reflect Fulcher's ownership interest	McCauley's Assumptions on Compensation Income Only, Adjusted to Calendar Year	McCauley's Assumptions of Fulcher's Share of Ownership Income (Loss), adjusted to calendar year	McCauley's Assumptions Including Ownership, adjusted to calendar year (the sum of column 2a + column 2b)	Fulcher's Adjusted Actual Post-Accident Compensation from Novitherm
2004	\$108,481	\$110,000	(11,917)	\$98,083	\$ 7,000
2005	111,085	132,000	(51,271)	80,729	7,875
2006	113,085	151,200	47,844	199,044	18,000
2007	116,364	166,320	166,815	333,135	20,250
2008	118,691	182,952	164,176	347,128	45,125
2009	120,234	186,873	93,464	280,337	97,500
2010	122,639	172,977	104,651	277,628	112,000
2011	124,969	172,067	148,271	320,338	98,500

Chart 5					
	1	2a	2b	2c	3
Year	Hoare's Assumptions on Compensation Income based on statistical evidence for employees and including zero to reflect Fulcher's ownership interest	McCauley's Assumptions on Compensation Income Only, Adjusted to Calendar Year	McCauley's Assumptions of Fulcher's Share of Ownership Income (Loss), adjusted to calendar year	McCauley's Assumptions Including Ownership, adjusted to calendar year (the sum of column 2a + column 2b)	Fulcher's Adjusted Actual Post-Accident Compensation from Novitherm
2012		189,273		312,818	98,500

[248] While I have not adopted either McCauley's or Hoare's approach, bearing in mind the foregoing and considering both lost compensation for services and lost ownership benefits, and bearing in mind my earlier conclusions about McCauley's and Hoare's methodologies and approaches, I have estimated Fulcher's gross past loss from the date of the accident at \$800,000 (before any deduction for contingencies.)

[249] To apply the statutory formula (80% of net), I have used the rough methodology used by counsel during argument (80% of 80% of gross: 80% of \$800,000 = \$640,000; 80% of \$640,000 = \$512,000.) On this basis, I have assessed past earning losses (80% of net) at about \$512,000 before the deductions for contingencies mentioned later in these Reasons.

Future Losses

[250] I have found that Fulcher will likely continue to work at Novitherm to the extent he can.

[251] It is difficult to predict the compensation Fulcher will receive for services to Novitherm in the future.

[252] McCauley projected Fulcher's 2012 compensation service-based earnings but for the accident at \$189,273. The Novitherm records indicate that his service-based annual compensation in the last two years has averaged \$98,500. It has averaged \$58,128 per year since the accident. Snyder gave evidence that Novitherm will pay Fulcher \$4000 per month + \$12,000 for technical assistance = \$60,000 per year in the future. This evidence together would suggest a loss of \$90,000-\$130,000 per year for lost compensation (apart from ownership income.) While I have found McCauley's estimates of compensation income too high, account must also be taken of lost ownership income.

[253] Apart from the 12% discount for business risk, McCauley did not make any reductions to his estimates of future losses for contingencies other than those related to his mortality and statistical non-participation in the workforce due to disability.

[254] Hoare's average statistical figures for employees with Fulcher's education in 2011 suggest average annual earnings for sales managers with his level of education at \$124,969, without including any loss for ownership benefits, a future loss for compensation income of about \$27-67,000 per year [plus something for lost ownership benefits.]

[255] Hoare provided an alternative method of calculating future losses. If one were to ignore Novitherm altogether and use his 2012 statistical earning of \$124,969 rounded to \$125,000, the future unimpaired loss to age 64 of an average employee with his qualifications in the workplace and reflecting statistical participation in the workplace would be $\$125,000 \times 6.5987 = \$825,000$.

[256] Rather than deducting projected actual earnings from Novitherm, if one were to use Snyder's 50% compromise figure, a 50% deduction for impairment using Exhibit 54 to calculate losses would result in a calculated future loss to age 64 of \$412,500. Or, if one were to use Fulcher's compromise figure of 75%, the loss to age 64 would be \$618,750.

[257] Again, these figures would not reflect that if the Novitherm business had been successful, he would have earned higher than the statistical figures paid to employees used by Hoare because he was an owner of Novitherm.

[258] I find a future loss (compensation + ownership income) of \$90,000 per year in 2012 dollars before any deductions for contingencies.

[259] Using Dr. Newell's evidence that but for the accident, Fulcher would not have needed the surgery causing his present disabilities until November 2022, a combined factor of 92.01 per 1000 would apply. To generate \$90,000 per year to age 64, the future loss would be $92.01 \times 90,000 = \$828,090$.

[260] Therefore, total loss of earnings/earning capacity to age 64 before any deductions for contingencies is $\$512,000$ past loss + $\$828,090$ future loss = $\$1,340,090$.

Contingencies

[261] In argument, counsel for the Plaintiff conceded there are applicable contingencies given Fulcher's vulnerable back before the accident.

[262] Dr. Shiffer said a person with a bulging or herniated disc is more vulnerable than someone who does not have that problem. I have noted her evidence that sometimes even coughing, lifting or any situation where a body is subjected to a sudden force or impact, can cause worsening of bulging or herniated discs.

[263] I have noted that after the jiu-jitsu injury in November 2002, Fulcher was keen to resume his various hobbies and activities.

[264] Dr. Newell originally said that given his active lifestyle, Fulcher likely would have needed the operation even if the accident had not occurred, "in five, ten or twenty years." While he eventually estimated Fulcher would need the surgery 20 years from November 2002, he found predicting the precise time frame difficult.

[265] If his active lifestyle had caused him to have surgery within five years after the martial arts injury, I have roughly calculated his damages for lost earning capacity to age 64 (80% of net) apart from other contingencies at approximately \$250,000. If he had had surgery 10 years from the martial arts injury, I have calculated his damages past and future for lost earning capacity apart from other contingencies at about \$512,000. If he had had surgery in 15 years, his past and future income loss would be about \$885,000. As I have already indicated, his loss to age 64 would be \$1,340,000.

[266] Dr. Newell gave evidence that whether or not he had surgery, given his vulnerable back and the possibility it would worsen, he might have had to retire at 60 or earlier.

[267] In all the circumstances here, especially the contingency that Fulcher's vulnerable back would have been further herniated and required surgery well before he was 64 (the age used in making my calculations), I would reduce my assessment for past and future earning capacity by 50% to \$670,000.

General Damages

[268] Counsel for the Defendant submitted that having regard to the negative contingencies and the possibility that he would have needed the surgery in any event, general damages should be in the range of \$50-75,000.

[269] I have found there is a significant contingency that Fulcher would have needed surgery well before 2022. Whenever he needed the surgery, he would likely have had the same pain and disability he now has.

[270] In all the circumstances here, he should actually receive general damages of \$100,000.

Per Quod

[271] Before I struck the jury, I ruled that the jury questions could include reference to a claim by Novitherm *per quod servitium amisit* (hereinafter "*per quod*.") My reasons are part of the Trial Record.

[272] Of course, Novitherm's eligibility to claim *per quod* damages is only the first hurdle. It must prove it has actually suffered such damages.

[273] Novitherm's *per quod* claim of \$186,738.02 for replacement services is set out in Exhibit 16. Counsel for Fulcher submitted that Novitherm is entitled to recover the entire cost incurred in an effort to mitigate its losses.

[274] Snyder gave evidence that in the fall of 2005, Novitherm hired a Mr. Michael Rogers to perform Fulcher's boiler room duties and to sell the Novitherm product. He left after one year. He was paid a salary of \$62,030.10 and expenses of \$6,049.31.

[275] In the fall of 2006 or early 2007, Novitherm hired Mr. J.P. Finet, Snyder said to make sales in Ottawa. However, on Novitherm's 2007 Organization Chart, he was shown as Utility Prog. Mass Markets USA.

[276] In March or April of 2007, Novitherm hired Mr. John Lower. He was paid \$1,785.59.

[277] When Lower left after two weeks, Novitherm hired Mr. Jason Ungerman in 2007 to make sales. He was dismissed after three months. Ungerman was paid \$14,747.39 and expenses of \$2,635.20.

[278] Neither Lower nor Ungerman made any sales.

[279] In early 2008, Novitherm hired Mr. Frank Faggiani to make sales in the GTA. He left after one year. He was paid \$51,538.40 plus expenses of \$6,211.29.

[280] In the summer of 2009 or 2010, Austin Fulcher was hired to computerize Fulcher's paper files. He was paid \$5,760 plus expenses of \$580.74.

[281] Counsel for the Defendant submitted that Snyder never explained the relationship between Fulcher's duties and those of the "replacement employees." He pointed out that after the accident, with the exception of a few months immediately after his surgery in January 2004, Fulcher continued to work in the GTA, his main sales area.

[282] I agree with counsel for the Defendant that Snyder's evidence on the *per quod* claim was vague. Had it been more detailed, I might have awarded greater damages under this head. Nevertheless, on the evidence before me, I allow the claim for half the costs/expenses for Rogers (\$62,030.00 + \$6,049.31), Lower (\$1,785.59) and Ungerman (\$14,747.39 + \$2,635.20).

[283] Obviously, greater expenses must be incurred to increase the capacity needed to increase income. As the number of employees increases, expenses will increase accordingly. This would have happened whether or not Fulcher was injured. I have allowed half of the claim for Rogers, Ungerman and Lower because Fulcher was back to work part-time and on good days was able to function relatively well.

[284] I do not allow the claim for Finet, who is described as working on marketing in the U.S.

[285] On Ex. 20, Faggianni is shown as "Regional Energy Advisor" and Young as "Senior Energy Advisor." I do not allow the claim for either "energy advisor."

[286] With respect to the treatment expenses of \$6,910.66, counsel for the Defendant conceded that they properly form part of Fulcher's claim and need not be included in the *per quod* claim.

[287] I allow \$43,623.75 for the *per quod* claim.

Housekeeping Expenses

[288] Exhibit 25 contains a summary prepared by Ms Cott based on the medical findings of Dr. Clements. She recommended four hours of housekeeping bi-weekly to 2021, when Fulcher reaches the age of 63, and four hours weekly thereafter to age 80; home maintenance

to age 70 of \$300 per season; window cleaning \$400 per season; car cleaning \$100 per year; car maintenance of \$316.17 per year.

[289] McCauley calculated amounts needed to fund these expenses. Those damages on Ex. 38.

[290] In my view, these expenses are allowable only so long as Fulcher would not have required surgery in any event.

[291] Given the contingency of early surgery, I assess these damages at \$30,000.

Cost to Finish St. John's Road

[292] The Plaintiff claimed estimated labour costs totalling \$60,551 to do work Fulcher was allegedly unable to complete on the St. John's Road property given his accident-related disabilities.

[293] Counsel for the Defendant submitted this Court should not allow these costs because Fulcher should have known when he purchased the home in 2005 [after the accident] he was incapable of doing that work.

[294] Counsel for the Plaintiffs submitted this claim should be allowed. This expense is conceptually no different from expenses for home maintenance necessitated by injuries to other plaintiffs. Fulcher should not be punished because of an extraordinary pre-accident capacity.

[295] I accept that before the accident, certainly before the jiu-jitsu incident, Fulcher would have completed the renovations himself. He did do some renovations after the accident that other injured plaintiffs might not have attempted. In principle, if the accident were the operative cause of the inability to do the work, the expenses should be allowed. I have found his back was getting better at the time of the accident and that he could have worked at Novitherm as before but for the accident. It does not follow that he would have engaged in the heavy lifting required to do those home renovations, given the continuing vulnerability of his back.

[296] I do not allow any damages under this head of damages.

Austin Fulcher's Family Law Act claim

[297] Austin was 12 at the time of the accident. He resided with his father about half of the time.

[298] Counsel for the Plaintiff submitted that Austin's loss in respect of his father's care, guidance and companionship was significant. Their recreational activities together have been eliminated or severely curtailed. Post-accident, it was necessary for Austin to assume responsibility for many of the housekeeping and home maintenance activities his father had provided pre-accident. He had to contend with a depressed, irritable and sleep-deprived parent. He suggested that a \$40,000 award under this head would be appropriate.

[299] Counsel for the Defendant referred to Austin's evidence that he and his father are as close emotionally as they were before the accident. The changes to some of their activities post-accident together have been for reasons unrelated to the accident. He submitted that Austin's *Family Law Act* entitlement is less than the statutory deductible of \$7500.

[300] I accept Austin and Fulcher have an exceptionally close bond, their pre-accident activities together have been seriously curtailed and that after the accident, Austin had to take responsibility for more housekeeping and home maintenance jobs [including kitchen clean-up, laundry, mopping floors, vacuuming, cleaning bathrooms, carrying groceries and lawn care.] While Austin's responsibilities in that regard would have increased as he got older, I accept that they increased much more than they would otherwise have done. While Austin and his father remain emotionally close, I accept that Fulcher's emotional problems have made those interactions less satisfying for Austin.

[301] I assess *Family Law Act* damages for Austin at \$25,000 gross minus the statutory deductible of \$7,500, for a net of \$17,500.

DISPOSITION

[302] Therefore the damages to Fulcher are assessed as follows:

For general damages:	\$100,000 plus pre-judgment interest
For loss of earnings/earning capacity	\$670,000
For loss of housekeeping + future maintenance.	\$30,000
For physiotherapy costs:	\$6,910.66

[303] The damages to Novitherm for its *per quod* claim are assessed at \$43,623.75.

[304] The damages to Austin Fulcher in respect of his *Family Law Act* claim are assessed at \$17,500 (net of the statutory deductible of \$7500.)

[305] Counsel may make submissions on costs on or before April 29, 2013.

M.A. SANDERSON

Released:

Citation: Fulcher v. Conklin, 2013 ONSC 2013

Court File No. 05-CV-293538PD1

Date: 20130408

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

EDWARD BRUCE FULCHER, AUSTIN
FULCHER AND NOVITHERM CANADA
INC.

Plaintiffs

- and -

CATHERINE CONKLIN

Defendants

REASONS FOR DECISION

M.A. SANDERSON J.

Released: April 8, 2013